

Wockhardt Ltd. (WOCKPHARMA)

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BSE Limited
Corporate Relations Department
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Scrip Code: 532300 National Stock Exchange of India Limited
Listing Department
Exchange Plaza
Bandra Kurla Complex, Bandra (E),
Mumbai - 400 051

NSE Symbol – WOCKPHARMA

Dear Sir/Madam,
Subject: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 – Investor Call Transcript
Pursuant to Regulation 30 of SEBI (Listing Obligation and Disclosure Requirements) Regulations, 2015 and further to our communication dated February 21, 2023, please find enclosed transcript of the investor call.
The transcript has also been uploaded on the Company's website and can be accessed through the following link:
https://statutory.wockhardt.com/stock_exchange_disclosures.htm
You are requested to kindly take it on record.
Thanking you,
Yours Sincerely,

Debashis Dey
Company Secretary

Encl.: A /a.

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Wockhardt Ltd .
Transcript of Investor Call
21st February , 2023

Snighter Albuquerque : Good evening everyone. And welcome to the investor Call of Wockhardt Limited to discuss the key developments of the company. We have with us Chairman Dr. Habil Khorakiwala and Managing Director Dr. Murta za Khorakiwala . I hand over to Dr. Habil Khorakiwala for his opening remarks. Over to you sir.
Dr. Habil Khorakiwala: I extend a very warm welcome to all of you. It is af ter a while , that we are having an investor's conference a nd we hope that we will maintain this communic ation w ith you on a regular basis. This communication today in a way, would be different than the normal investor's communication. Because what we would real ly want to share with you today i s that Wockhardt is in the cusp of significant changes. And I want you to know what is this significance turnaround situation that we expect i n next 20 to 24 mont hs. Our Managing director, Dr. Murtaza Khorakiwala will elaborate s pecificall y on these major achievement s, accomplishment s and the changes which will take pl ace and focus on major milestone we have achieved in recent past i n last 12 months, over to you, Dr. Murtaza .
Dr. Murtaza Khorakiwala: Thank you, Chairman and the very warm welcome to the investor communit y. I'm very happy to be here and share with you some of the changes that and the transformation that is undergoing in our organization and how we intend over the next 12 to 24 months to take that forward.
The first slide is just a snapshot of the organiza tion in the nine months in terms of our financials, revenue , EBITDA and debt. And moving on, this gives you a sense of the organi zational structure on slide 4, A global footprint of the organization where we are present outside India and in various m arkets and it contributes about 75% of our business outside India, 25% is in India and we are present in various segments like gener ics, branded , biotechnology , injectable a nd antibiotic discovery, I will share with you a little bit more as we go along into the new

developments that have taken place in our business and the promising growth opportunities that are there. Going on to the next slide, the key highlights I would like to share with you is on our performance that in the last year, what is the entire restructuring that we have done of the US business. And a great opportunity in terms of the agreement that we have with Serum in terms of vaccine manufacturing in UK. And what that may unfold in the coming year and obviously an update and progress into very significant progress we have made in our novel antibiotic portfolio.

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Next slide, we have been mentioning in over the last one year that the US business needs to get significantly restructured. And as it was having a significant amount of drain into the financial performance of the

company, we

have shut down our manufacturing facility at Morton Grove near Chicago and using a simple 80-20 formula, we have identified the product portfolio to be manufactured by a third party and that will be an ongoing continuing business that we will have. As a result of the entire restructuring of the manufacturing, we intend to save about \$12 million in losses which we are currently incurring and however we will continue to maintain our sale in the US business as these products will be manufactured from third party certified facility with approximately 40% gross margin.

Moving on to the next slide, we have recently concluded an agreement with Serum Institute for manufacturing vaccine in the UK facility earlier this year. And as a result of that we have received £10 million as a contribution for reserving manufacturing facility for 150 million doses per year of vaccine for 15 years. This is 150 million doses per year for about 15 year agreement. And in addition to a contract manufacturing, there is a joint venture component in the agreement, where there is a profit share of 51:49 in favor of Wockhardt. Today, Serum has identified 2 vaccines and we plan to manufacture these products within the next 12 months after exhibit batches and regulatory approval is received.

We have been significantly committed to our presence in antibiotic over the last 20 years and are probably the only company in the world to have a very comprehensive end to end drug discovery and development program in antibiotics. As you already know, we have about 6 QIDP grants from the US FDA and this means that there is an unmet medical need identified by US FDA and being satisfied by our product profile, therefore they have given us a QIDP approval, which will allow for faster clinical trial and approval of the products.

I share with you a pipeline of the presence of Wockhardt with five products in the development pipeline vis a vis, other entities and other corporations who have between one to two and this just goes to show the strength of the portfolio that has emerged out of our dedicated commitment in research over the last 20 years, positioning us in a way good position as far as antibiotic innovative portfolio is concerned.

We have achieved the success in licensing one of our molecules for China, which you are aware is a licensing deal for China with Jemincare 4873, we have received the milestone payment from Jemincare. As far as our India approval of phase three, we are marketing two products Emrok and Emrok O in India and we are also filed for approval in various emerging market and expect to receive approval in eight emerging markets in the next 6 to 9 months. In India, over the last two years that it has been present, 30,000 patients have used Emrok and benefited from it.

Additionally, our phase three clinical trial for 4873 is going to be concluded in the Indian market some time in 2024. Let me tell you a little bit about our flagship program that is WCK 5222. And there have been a couple of significant

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milestones and developments in this regard. First, there is an increase in the resistance. A new report by CDC in the US hospitals concluded that the resistance to infection on which 5222 works has increased somewhere between 30% to 80% because of two years of COVID-19. This further enhances the potential and makes 5222 even more relevant as a lifesaving medicine. Our global phase three trial which commenced in August 2022 is progressing well and we intend to complete this trial over the next 15 to 18 months and seek approval in markets globally in the US, Europe, China & India and be in the markets sometime in 2025.

You will be happy and proud to know as an investor in Wockhardt that your company has the innovative 5222 product already saved 3 lives of patients in India, even before it is approved and undergoing

phase three trials. 50

year old female patient in Vedanta Hospital who had abdominal sepsis and 11 year old boy in global hospital in Chennai who had a bone marrow transplant and an 18 year old boy admitted in AIG Hospital Hyderabad with acute leukemia, 5222 was used on a compassionate basis for saving the lives of these three people when all other medicines that were used by the doctors could not work. It is indeed extremely fulfilling and proud moment for Wockhardt and for all Indians. These were the critical patients who were on ventilators for several weeks, they were cured and discharged from the hospital on completing treatment in 10 days with WCK 5222. And this is indeed beta lactam enhancer and is a new class of antibiotic and therefore it works so dramatically and with a very superior clinical benefit.

Additionally, you will be happy to know that we have established a collaboration with the National Institute of Health

in the US, with one of our other lead molecules 677. They are conducting a human phase one clinical trial of MDR gram negative antibiotic targeted for ambulatory settings, this indicates NIH confidence in the novel once a day, much needed outpatient parenteral antimicrobial therapy for MDR infections in an ambulatory setting.

These developments of the organization over the last 12 months provides a glimpse of the future of your company. It is like an iceberg which you see and are able to see a very small portion. But there is so much underlying strength and opportunities unseen and invisible today. I'm not getting into all the financials which have already been published and I'm sure most of you have seen them. However, I would like to highlight few important and significant aspects. One of the objectives which we had was in terms of deleveraging our organization. Our long term debt external debt over the last five years has gone down from Rs. 3,200 Cr to about Rs. 600 Cr. In terms of our external debt to equity, it has gone down from 0.96 to 0.16. In parallel to deleveraging our organization, our promoter commitment has increased and it has improved from Rs. 208 Cr to Rs. 700 Cr and which could be considered as a Quasi-equity. In addition to the outstanding Rs. 700 Cr loan, another Rs. 500 Cr of promoter loan has been converted into equity and we have received an additional Rs. 250 Cr from investor in the successful rights issue.

Coming to some of the operational parameters, our sales has shown in the current year a quarter on quarter, increase in growth from Rs. 578 Cr to Rs. 699 Cr representing growth of about 20% in Q3 over Q1FY23. Our

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EBITDA also shows a steady improvement and has moved in the positive territory and for the last two quarters is positive with Rs. 39 Cr and Rs. 59 Cr EBITDA. Our cash flow position shows that our operating cash flow in the Q3FY23 was a positive Rs 95 Cr. This helps in deleveraging the organization by reducing debt by about Rs. 109 Cr. Additionally, the promoters infused another Rs. 71 Cr in terms of a cash flow which helps in taking care of the investments that we are making in our R&D and NCE portfolio. And therefore, if you see right from the beginning our opening balance, which is Rs. 221 Cr remains at about Rs. 217 Cr for the end of the quarter.

Let me come to the highlights of some of the individual businesses and their performance in the current year. Our UK business without the vaccine has moved up in terms of its growth from Rs. 208 Cr last quarter last year to Rs. 223 Cr, showing a handsome increase in the turnover as well as an increase in the market share in the covered market by more than 1%. We have accelerated our focus in developing and filing new products and as you would see since 2020 in a combined way, we have filed more than 23 products and another 24 products to be filed in 2023 and 2024.

Our India business continues to per

form well with a focus on diabetes and Emrok and a pain management portfolio.

On a 9 month basis a diabetes business has grown by more than 20% and on QoQ basis from Q2FY23 to Q3FY23, there is a growth from Rs. 150 Cr to Rs. 175 Cr and on 9 monthly basis from Rs. 450 Cr to Rs. 483 Cr. And we have one of the few companies in India that has a complete integrated portfolio of diabetes products, including insulins, glargine s and fully integrated guide from R&D to manufacturing to commercialization.

Emerging market business has also shown an improvement and growth in Q3 compared to the earlier quarters and stands at Rs. 148 Cr compared to Rs. 117 Cr. Our distribution among the key markets or the key regions where we are present is Latin America, Asia and Russia CIS. And going forward MENA will become also a very important market in terms of our growth.

Having given you an update on the various businesses and their performance, let me share with you a little bit about how we see the various drivers of growth driving growth for the organization in the years to come. And as you are aware, fundamentally we are focused in four areas which we call as the strategic pillars of growth. First is the main stay pharma business which is there in India, UK, Ireland, US and emerging markets. Second is the vaccine opportunity, which we realized from our collaboration with UK government and now an ongoing collaboration with Serum for manufacturing these vaccines in UK and supplying globally. Third very importantly, is our presence in the biological space in diabetes, where we have the entire injectable insulin portfolio, we have marketed 2 products and additional products are under development and we have introduced in India and various emerging markets and intend to expand this opportunity in terms of markets and products. And overall these targets are diabetes, biological market of about \$50 billion. And lastly, but not the least, is our foray and presence and deep commitment in this R&D space of new drug discovery, which I mentioned to you brief a while back.

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If you see what are these growth drivers, how they will unlock value and performance in the organization going forward, I see in the first two years that is in a one or two year time frame we will have these diabetes, insulin, glargine s and various molecules in emerging market driving a large percentage of the growth. Our collaboration for vaccines with Serum, which is there and which we have signed will deliver the growth in the year. And as an organization, I think our fundamental focus is on improving profitability and cash flow. I think all aspect of the operation is something we are looking at and we have been focused on it in the last year or so and we'll continue that focus in the years to come is on improving our profitability and cash flow. Going forward in a two to three year time frame. Our novel drug discovery portfolio, which we have launched in India and some other new products which are coming will also help in driving some of the growth of the organization.

Let me elaborate a little bit on the growth drivers and what is it that is, how it will drive. I think we are in the insulin and glargine space in India and emerging markets, which is responding about USD 1.5 billion in terms of the covered market, our current presence in this space is about USD 50 million and therefore, there's great opportunity for us to increase our market share. We are registered in more than 25 markets and a large number of new markets we have filed and we have approvals in the last year and in the coming year and therefore it will accelerate our presence with new market launches. It is a limited competition and therefore has the ability to create and develop a sustainable competitive advantage, and I will share that with you in a while as to why this can be a sustainable

competitive advantage, we are fully integrated in terms of our manufacturing right from API to formulation to having our own devices and our own IP.

We believe that being integrated right from beginning to ending with R&D, manufacturing, regulatory, trend devices and having our own IP and commercial organization and having an end-to-end kind of a value chain, it provides us a very sustainable competitive advantage in terms of cost, in terms of value creation, in terms of an end-to-end integration of all effort and execution and operational. In addition to the existing products, we also have a pipeline of new products including Insulin Aspart and various other analogs which are in the development pipeline.

I touched upon this earlier, but our collaboration with the vaccine with Serum is for 150 million doses per year. It is a multi-vaccine arrangement including COVID and it is a long-term 15-year profit sharing arrangement in addition to the CMO arrangement which is there. I had mentioned about COVID and its impact on increased opportunity for 5222 and you would be surprised to know that COVID-19 in the two years that it was a pandemic resulted in the loss of 5 million lives. And the Super bugs which we call as the resistant organisms which are resistant to current antibiotics and result in the loss of life of 5 million people every year. And that is an astounding fact when one looks at it in the context of the pandemic and almost it is like a silent killer.

This is our overall portfolio of products that we have in antibiotic space and we cover from gram negative to gram positive as a first-line therapy to life-saving product. The entire spectrum of products. Covering different indications and there are different stages of clinical development somehow launched in India, filled with emerging markets, and

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two of them, 5222 and 4873 are undergoing phase three clinical trials. So let me finally conclude my communication and share with you from where I covered. What are some of the significant milestones we have accomplished in the last one year? What is the financial performance of the company in the last year? What is broadly the strategy and the growth drivers that we are looking at and now specifically what are the specific milestones which we are focused on in terms of delivering these in the years 24 and 25 and these are.

We will get approval of glargine in some of the key Latin markets like Mexico, Brazil and in MENA. We are going to file our Aspart in India. We will get approval of Emrok and Emrok 2 in the emerging markets. We will complete our phase three clinical trial in India in 2024 and file for approval. We will complete significant part of the phase three clinical trial of 5222 which is being conducted globally and will be filing for approval in 2025. We will initiate our business through the collaboration with Serum and manufacturing in UK. Our US business has already moved and we structured to a third party and we will initiate product supply from these third party shifted products. We are targeting new product launches in India as well as in various other markets. 14 launches in India and 25 launches in UK in 24-25 combined. In 2025 we will launch Aspart and 4873 in India. And by then our US business, which has been a drain on the profitability and cash flows will turn profitable, definitely. As an organization we are working towards clearly being PBT positive in two years' time. I think this was a brief communication we wanted to share with you that Encapsulates, as chairman said some of the very important and key milestones, but more importantly the key areas we are focused on in terms of delivering a better performance and improved financial health in terms of our profitability and cash flows. Thank you for being with us and partaking in this presentation communication. And I would like

have the floor open now to any questions that you may have.

Snigter Albuquerque: Thank you, Dr. Murtaza. I request our investors to please take to the chat box or the Q&A box to ask any questions that you may have and we will read out your questions and take them accordingly in a systematic manner. We will give it a few seconds before our investors tag their questions in the chat box. Our first question is from Kishore Aggarwal. Can you briefly help us understand what will be the total cost yet to be incurred in respect of WCK 5222 and how does the company plan to fund it?

Dr. Habil Khorakiwala: I think we are already initiate d phase three. And we needed to complete it next 12 to 15 months. And the additional investment required for clinical trial will be about USD 30 million. And we intend to not take out the cash flow from our operation, we are looking at alternate funding, debt funding and some monetization of assets which are not in use. So that's how we intend to fund balance of 5222 Clinical trial.

Snigter Albuquerque: Thank you, Sir. The next question is from Mr. Harish G. What's the revenue growth expected in FY24 and the cash EBITDA in FY24?

Dr. Murtaza Khorakiwala: While we don't give any future guidance, but I must say that I have been fairly heartened by our performance in the last nine months and the growth momentum that we have bought in terms of

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our profitability and we have initiated various measures not only on the top line growth, but I think what our focus is on driving profitability. What we have looked at is all aspect of the business we have looked at product wise profitability. We have looked at cost management aspects within the organization and working capital. So I'm very positive that we will maintain the growth momentum that we have gotten this year and it will only accelerate in the coming years. As I mentioned, in terms of the various growth drivers we have, we are having new launches of our diabetes portfolio, insulin and glargine in various emerging markets. Like in MENA, Mexico, Brazil and various other Middle East markets, our new filings in the UK is on a good path, we have had 12 filings in the last year and another 25 filings we'll have in the next two years. So I think all aspects of business are on a right track. We are going well, US business, which was a drain we have restructured and as a result of that, it would bring about a saving in terms of our bottom line because the saving of USD 10 million that we hope to realize, part of it has been realized in this year, significant amount would be realized in the coming year. So all this put together. I think we would see significantly better numbers in the next year and then we will turn completely PBT positive in 2 years from now.

Snigter Albuquerque : Thank you, Dr. Murtaza. The next question is what is the latest status for WCK 5222 trials and how many patients have enrolled for the same?

Dr. Habil Khorakiwala: As we already mentioned that WCK 5222 phase three clinical trial is on, we have enrolled patient in Europe and very soon we will enroll patient in India, US and China. And about 20% of the patients have already completed the clinical trial and as I mentioned in next 15 -18 months, we should be completing the clinical trial on 5222.

Snigter Albuquerque : Thank you, Sir. The next question is what will be the peak revenue from vaccine business related to Serum?

Dr. Habil Khorakiwala: See as per our agreement and Serum's understanding, we have reserved 150 million doses capacity. Obviously there are two aspects of income coming out of it. One is we will receive a normal manufacturing cost of making those vaccines as we were receiving earlier from the UK Government. Additionally, depending on the price at which Serum will sell after their marketing cost of 8% to 10%, we will be sharing in the profit.

it. So obviously per dose there is a double value creation. One is a manufacturing link, another is a link and we expect Serum that is what they have said, they are starting with two vaccine within next few months after agreement and I'm sure as we go along, they will receive more license and they will utilize our capacity and I think it will be a game changer for us in terms of value creation and profitability over next several years and it would be a very sustained increasing business year after year.

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Snigter Albuquerque : Thank you, Sir. We go to the next question. What is your expected R&D spread over the next two to three years and how do you intend to fund it? Also if you could share how much you intend to spend for Biosimilars and NCE?

Dr. Habil Khorakiwala: Our R&D funding if you have observed that we have maintained certain percentage on our revenue of the same, but within the R&D the mix has shifted away from pharmaceutical R&D to Drug discovery and biological R&D. So our major part of the spend comes out of biological and drug discovery R&D and we intend overall to maintain 8% to 9% of our revenue for R&D.

Snigter Albuquerque : Thank you, Sir. The next question is what is the status of US FDA on our current plans?

Dr. Habil Khorakiwala: See all of us in the industry know that US business has become quite challenging for variety of reasons and as a result of this, we have restructured our manufacturing at MGP level. We are also revisiting our India business, from India to US and currently our focus primarily is on the third party manufactured product to sell in USA and probably we will take in due course of position of exactly how we want to deal with our US opportunities and the business in the coming year and once our plan is ready we will be happy to share that with you.

Snigter Albuquerque : Thank you, Sir. We go to the next question, how will the product mix change in the next two to three years from current levels?

Dr. Murtaza Khorakiwala: As I was mentioning our focus on Biologicals and Biosimilar is only going to grow and expand its presence as a part of the overall business. So currently biologics contribute about 20% of the whole revenue of the organization. It will grow at a much faster rate than the generic business and I expect that in the next two to three years, its contribution to Wockhardt business will improve from 20% to 30%. Primarily driven by the launches of the products in various emerging markets and the new products that we are developing that will come to the market in a medium term of time, but the existing products and the launches in new market increase in the market share that we have, all that would drive growth of biologics. So I think that is one aspect of product mix. Second is I think as an innovative portfolio and NCE portfolio, which is approved in India and we launch in various emerging markets, as that comes into play, that part of the business and portfolio will also increase. But in the first two to three years, I see diabetes, the biologics part of it having a very significant impact.

Snigter Albuquerque : Thank you, Dr. Murtaza. The next question is, is it possible to share the centers in India where the trial of 5222 is being conducted?

Dr. Habil Khorakiwala: We have not yet started in India, the 5222 it is now mainly in Europe. But I think next few months we will initiate this in Indian centers also. And we will let you, I think it will be available in our website

whenever we initiate a trial in India, but it is currently it is not yet started.

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Snighter Albuquerque : Thank you, Sir. The next question is, is there any plan to out license AMR molecule s? And any plans to monetize WC K 5222?

Dr. Habil Khorakiwala: See, we basically have to one way or the other monetize 5222 and there are two aspects

of it, because 5222, we are conducting global clinical trials and we do not have presence in many markets in the world. So that is where we will have to monetize. We are also looking at an option of globally to monetize the product and I think these are the we are evaluating the value proposition, which is available, which is possible for us and that is also an option on the table very much and we will take a call at the right time after the phase three clinical trial is over.

Snighter Albuquerque : Thank you, Sir. The next question is, could you share broad monetization potential for WCK 5222 for the company?

Dr. Habil Khorakiwala: I think you put a number at this stage is a little too early. I think we'll have to wait for a while and then make a proper assessment. But I think it will be a fairly significant number.

Snighter Albuquerque : Sure, Sir. We go to the next question, what are your thoughts on biosimilar business? The market is already competitive with multiple players in the emerging markets area. What are your thoughts on pricing etc?

Dr. Murtaza Khorakiwala: As I mentioned, I think in my communication there are three of important aspect of our presence in emerging markets in India in a biosimilar space and the components of that strategy are the following. One is we are fully integrated player in a biosimilar space, right from R&D to manufacturing to commercialization and there are not too many players in the world who have that complete integrated play and that provides us the cost competitive advantage, sustainability advantage and the commitment of the organization in a biosimilar space. It's not a space where you have 15 players and 20 players. It's a space where you will have 4-5 players, which appears as they are.

Dr. Habil Khorakiwala: This is very true for a diabetic portfolio. If you look at other biological products, the competitive scenario is much larger. Many more players are there, but when you look from a diabetic portfolio of insulin, glargine, Aspart, it's quite limited in there. Only half a dozen players worldwide.

Dr. Murtaza Khorakiwala: And in addition to the technology aspect in diabetes biosimilar, I think the fact that one has to put up very specialized manufacturing facilities, API as well as formulation and the kind of investment required and the time that is required to put up the entire facility, get regulatory approval takes a long period of time, so I feel that in fact we are in a very sweet spot as far as the diabetes injectable portfolio is concerned and we will be looking at very significantly unlocking potential in the market and we are very well positioned.

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Snighter Albuquerque : Thank you. The next question is, did we proceed to phase three directly without phase two? I could not find any details on phase two study. I assume that it's allowed to do phase three without phase two. Is it based on certain factors related to WC K 5222 which gives us confidence that it will be successful possibly without the need of phase two?

Dr. Habil Khorakiwala: See, this whole program was shared by us to US regulators, EMEA regulators, Chinese regulators and even Indian regulators and the data which we were able to produce on a preclinical and phase one data. They were so strong in terms of safety profile because phase two generally you do for safety to a large extent and efficacy also, but phase three is fundamentally both safety and efficacy. So looking at the data and also the very fact that needs are there and the safety profile was excellent, both in terms of preclinical phase one clinical and also with the various organs like heart, lung, liver, kidney. So we did a safety profile and all the organs and the data in terms of safety was outstanding actually. And therefore the regulator have made an exception and granted us straight phase three. The second, equally important

aspect is Cefepime is already an approved drug. Zidebactam is a new discovery molecule by Wockhardt, and as a result of that, overall they felt that we can straight away move into phase three even if you have noticed in phase three, they have asked us to do one ARM study only. Normally, for a new molecule it is required Two ARM study to establish. So all these advantages we got mainly because of the preclinical data. And phase one safety data. And additionally the unmet need and we have already seen with the compassionate use which have taken place in India even though our clinical trial is for C-UTI, all these products were used which we would be taking HAP/VAP study that is ventilator study, when person is on ventilator but for all the patients where compassionate use was done. There were cases of lung infection. Some of the Organism which we are talking of, Pseudomonas Acinetobacter, they were involved. And they recovered like magic, on 3rd day there were in one case, ventilator was off. In all cases at therapy on completion, there was a complete remission of all the resistant Organisms.

Snighter Albuquerque : Thank you, Sir. The next question is on the Serum deal of 150 million doses per annum. Has serum guaranteed you minimum doses and what should be the pricing? Also what kind of overall margins can

we see in this business?

Dr. Habil Khorakiwala: See, this is a deal which we have and there is a certain minimum quantity which is guaranteed after initial phase. That is one, second we expect significantly more margin per dose of vaccine compared to what we did for COVID-19 vaccines with the UK Government because what is profitable, we'll get our manufacturing cost. Additionally there is a profit sharing.

Snigter Albuquerque: Thank you, Sir. The next question is how much revenue from Emrok and Emrok O sales are that we report in nine months, FY 23 and what are the revenue prospects going forward?

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Dr. Murtaza Khorakiwala: So in the last few years since we have launched, we have been clocking upwards of Rs. 30 Cr in a financial year. And if you look at in terms of the number of patients who have benefited from Emrok, we have served 30,000 patients since its launch. It has also been used in various cases of COVID where patients had pneumonia and they developed resistant infections like IMSA infections. And in fact, some of the doctors even mentioned to us that it was useful to have Emrok launched right during COVID time because it helped them in dealing with some of the pulmonary and lung infections. More adequately, the clinical performance of the product has been remarkable in India, and there's been a very positive appreciation and also adoption of the product, from a clinical point of view. And we have, as I said earlier, we have filed the various emerging markets and we expect to get approval in about 8 emerging markets in the current year and we will expand that activity to a larger number of emerging markets in the following year.

Snigter Albuquerque: Thank you Dr. Murtaza. The next question is. Could you please benchmark efficacy of WCK 5222 versus other antibiotics recently approved and under trial.

Dr. Habil Khorakiwala: Two things. One is very clear, there is no recent approved antibiotic which works on highly resistant Pseudomonas, Acinetobacter Metallobactam, the one which works on KPC and OXA 18. That is common. The other three are very unique to WCK 5222 even existing works there are report that Avycaz which has got off patent recently and getting marketed in India has already there are reported developed resistance against Avycaz. So from that point of view, there are no drugs in the market which comes anywhere near 5222. At the same time, we don't see any drug in the clinical trial which are there in covering the spectrum. The only other drug which covers the spectrum

among the new drug is Shionogi drug, but it has such side effects and it has received a black box warning for usage in the US.

Snigter Albuquerque: Thank you, Sir. We move to the next question. Wockhardt has reduced external debt to Rs. 600 Cr. What are the total funding requirements over the next two to three years both on CapEx and R&D in the clinical trial space? And how do you intend to fund in this?

Dr. Habil Khorakiwala: I think I may have answered the question. We expected about USD 30 million of 5222 CapEx we require in next to complete the clinical trial and we are looking for external funding for that purpose basically. And there are no CapEx required for manufacturing because 5222 we have derisked from manufacturing and we are getting it manufactured both API and formulation in approved FDA approved European facility and that is where the clinical batches are being manufactured for clinical trial and those are the companies where we will continue to manufacture commercial usage.

Snigter Albuquerque: Thank you, Sir. The next question is particularly with respect to Serum, with COVID on decline are we confident of the 150 million dosages?

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Dr. Habil Khorakiwala: The whole issue is Serum is not looking at COVID as a vaccine. They are a very broad based vaccine manufacturer. Even before COVID was introduced, nearly 50% of world production of vaccine was done by Serum and they have such a strong dominance in the world. Any new vaccine which gets developed, Serum becomes one of the very natural partners for anyone to get manufactured and they have the capacity to manufacture API vaccine. Our arrangement with them is to manufacture the formulation for them in UK so that they have an access to many markets other than WHO markets. And also they get a strategic advantage of getting manufactured in UK of getting the partnership licensing arrangement. So one is not really looking at the COVID extension. But there are many other vaccines which are coming. They are highly valuable vaccine in terms of price and other thing. And that is where the collaboration is what they expect to happen.

Snigter Albuquerque: Thank you for moving to the next question. While closing your manufacturing in the USA, are we also cutting down on our sales?

Dr. Habil Khorakiwala: I think what we have taken our call is in USA we had a manufacturing standing course about USD 25 million. What we have done is we have identified few products which contributed to more than 80% of our top line and more than 150% to 200% of our bottom line. Because these are high margin product and that is where we are getting it manufactured in US and Canada as a third party manufacturing product. So we have really not sacrificed too much of sales. At the same time, we have significantly increased our profitability of our operation in US and the losses which were you are entering that gets covered and we expect that overall margin of our US operation will be about 40%.

Snigter Albuquerque: Thank you, Sir. That was the last question for the day.

Dr. Habil Khorakiwala: So let me just conclude this investor conference. Basically, I'm so happy that so many questions were asked and we had an opportunity to clarify. Dr. Murtaza, our Managing Director has given you very clear perspective of where we are today, where we will be there next 12 months and we will be there in next two to three years. And to just sum it up, I personally feel we are at a very important point in the life of the company and we are at a turning point stage. We went through challenging times in the past. But where we are moving forward is opportunities, opportunities and opportunities. Opportunities in biologics globally, opportunities in vaccine manufacturing with Serum. We are also in discussion with another large company for making their vaccines into our facilities. So

UK is going to become an important source of revenue and profits. And most equally importantly, that we are getting very happy that 4873 will be completed clinical trial and launched next year in the other markets, Emrok is getting traction and obviously 5222 we are seeing that clinicals will be over, and we will try to see how to monetize the value coming out of 5222. With these thoughts thank you very much for being with us and wish you all very best, and even though it is late, I can still wish you all a great 2023.

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Snigter Albuquerque: Thank you so much. Sir and we wish you have a good evening. Please if you have any further questions, you can reach out to the Investor Relations team at Wockhardt or at Ad factors. We would be more than happy to take all your questions and address each and every one of your questions or E-mail or set a meeting with the management. With that I hand it over to Dr. Murtaza for his closing remarks.

Dr. Murtaza Khorakiwala: I would also like to thank all of you for being a part of the investor conference. And as chairman has summed it up very nicely. We are looking at opportunities, opportunities and opportunities in all areas and we are at a turning point and we hope to come back to you sequentially and deliver on the performance and the results that we have very clearly identified over the next 12 to 24 months. Thank you once again and a very good evening to you and have a great 2023.

Snigter Albuquerque: Thank you, Dr. Murtaza. With that, we end today's investor call. Thank you everyone and have a pleasant evening.

Statutory Declaration: Certain statements in this call may be forward-looking statements. These statements are based on management's current expectations and are subject to uncertainty and changes in circumstances. These statements are not guarantees of future results.

Note: This Transcript has been slightly edited at few places for clarity and accuracy.

Call: Jun 2025

Ref. No.: WOCK/SEC/SE/2025-26/015 11th June, 2025

BSE Limited
Corporate Relations Department
P J Towers
Dalal Street
Mumbai - 400 001
Scrip Code: 532300 National Stock Exchange of India Limited
Exchange Plaza
Bandra Kurla Complex
Bandra (E)
Mumbai - 400 051
NSE Symbol: WOCKPHARMA

Dear Sir/ Madam,

Subject: Disclosure under Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended – Transcript of Investor presentation.

Pursuant to Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended and in continuation to our letter bearing reference no. Ref. No.: WOCK/SEC/SE/2025-26/012 dated 2nd June, 2025, please find enclosed the transcript of the presentation made in the Investor Meeting held on Thursday, 5th June, 2025, for your information and records.

The said Investor Presentation Transcript will also be uploaded on the Company's website and can be accessed through the following link:

<https://www.wockhardt.com/investors/analyst-investors/transcript/>

Kindly take the same on your records.

Thanking you,

For Wockhardt Limited

Rashmi Mamtura
Company Secretary

Encls: as above

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"Wockhardt Limited
Investor Conference "
June 0 5, 202 5

MANAGEMENT : DR. MURTAZA KHORAKIWALA – MANAGING
DIRECTOR – WOCKHARDT LIMITED
DR. HABIL KHORAKIWALA - FOUNDER CHAIRMAN —

Moderator: Ladies and gentlemen, good day and welcome to Wockhardt Investor Conference. We are pleased to have investors joining us both in person at the NSE Exchange Plaza and virtually from across locations. As per today's agenda, we will begin with a presentation by Dr. Murtaza Khorakiwala, Managing Director of Wockhardt Limited. This will be followed by a Q&A session open to both in-person and online participants.

Please note that this session is being recorded. We request members of the media to kindly reserve their questions until the investor segment is concluded, after which the floor will be open for media interaction. As a reminder to all online participants, your lines will remain in listen-only mode throughout the session.

With that, I now hand over the conference to Dr. Murtaza Khorakiwala. Thank you and over to you, Dr. Khorakiwala.

Murtaza Khorakiwala: Okay. So, thank you and I would like to welcome the investor community for sparing your time and being with us today. Today marks also the celebration of our 25 years at the NSE and since it is the end of the last financial year and the beginning of the new one, we thought it would be a good time to share our progress and an update on the performance and the outlook of the company over the short and medium term. So, thank you for being with us. I'll take you through the overall perspective of the organization today, the performance over the last year and what is our plan going forward in the next 2 or 3 years.

So, we are a global research-driven multinational company. We started off as an Indian company 55 years back in a generic business and over the last 25 years, we have evolved into a global company where 70% of our revenue is outside India. Also, our focus and our operations have fundamentally moved or are moving from being a generic focus to a research-driven organization, which includes our foray into novel antibiotics which we started about 25 years back and biotechnology.

And jointly these research-driven businesses contribute about 18% of our sales turnover today. For the last year, we had a turnover of INR3033 crores and we had an EBITDA of about INR418 crores. The 80% of our business today yet remains a pharmaceutical business and that is spread across various geographies which I will share with you as we move along.

So, we have an established global footprint and that is from a competitive point of view, it allows us to access a wider market beyond India. We have 12 manufacturing facilities in various parts in India and UK in Ireland and two R&D centers in India and UK. 23% of our business is in India, 77% is outside India mainly in UK, Ireland and emerging markets. In UK and Ireland, we are among the top five generic companies in those markets and we hold a strong position over there.

So, coming to our performance in the last year has been a good and strong operational performance we have had in the last year. Our EBITDA has grown by 67%. Our biosimilar business has grown by 20%. Our net debt to equity now is only 0.01% and over the last year we

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have reduced our debt to INR326 crores and that was helped by the QIP which we had of INR1000 crores about 6 months back, 9 months back.

So, let me take you through some of the highlights and the positives that we have achieved in the last year and the most exciting from an investor point of view is the work we are doing in our NCE portfolio which holds the most promise, which is unique, which is specific to Wockhardt and over here we are world class and world leaders in the discovery space.

So, we have various molecules in our NCE portfolio and achieved a lot of progress and update in these areas. So, as far as ZAYNICH is concerned, during the year we completed our global Phase 3 clinical trial and we had out-

standing results where we had a 20% superiority to the existing treatment which is given.

Normally, a drug you have a non-inferiority clinical trial to get a 20% superiority in a product is truly outstanding and similarly we did a second study in India which was a Phase 2 study against carbapenem resistant organism where the product where the patient has organism which are resistant to the existing antibiotic and after that ZAYNICH is given and in those patients also we had greater than 90% efficacy.

So, and also we have filed with the DCGI on in March 25 . During this 1, 1.5 year period when we have been doing Phase 3 clinical trial a lot of clinicians have asked about of the product to us to treat patients who are untreatable by existing treatments and as India and various countries around the world have a compassionate use facility where if the clinician feels that for the betterment of the patient to treat the patient if there is an unapproved drug but it has a promise they allow to give that drug but there is a regulatory system around it.

So, we have saved 51 lives in the last 1.5 year by having ZAYNICH given to patients who are otherwise untreatable by existing treatments . And that just goes to validate in real life the results that we have got in the clinical trial of 20% superiority and 90% efficacy in case of resistant organisms.

The other molecule that we have that is Mignaf , has been approved -- during the last year has been approved in India and now launched in India on 27th May 2025 and we have developed a specific business unit to take this product to the market to the specialist. I'll discuss that a little bit more in detail as we go along.

Also during the year , we have been recognized and received a breakthrough medicinal product category by Saudi Arabia recognizing the unmet medical need that is there in the treatment of pneumonia and how our product satisfies a unique medical need and by giving a BMP approval that facilitates the approval and the use of the product in the market to treat untreatable pneumonia infections.

We have another product called Odrate , and Odrate is very unique in its way that normally the injectable products which are there is given twice a day or thrice a day and this unique product is given once a day . And therefore that greatly simplifies the treatment and allows also for the potential for use of the product outside of hospitals in a day care facility and greatly reduces the burden of hospitalization and insurance cost on the patient.

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So this has been actually picked up by NIH , that's the National Institution of Health in the US seeing the potential of the product and promise and they sponsored and funded the Phase 1 clinical trial in the US which has been done and the study is over , the Phase 1 is over , results are good , and so that we have completed in the current year.

We also have another product which is 4282 : Foviscu and where we are completing the phase - of the Phase 2 clinical trial. This is another product for hospital and in the gram negative space where ZAYNICH is at the top end of the pyramid. This would be at an entry level product and the usage of this would be very wide and would cover a large base and would be the first drug of choice for gram negative infections in the hospital.

We launched EMROK in India 3 years back and just after COVID and over the years we have treated 100,000 patients successfully and achieved a market share of about 10 % over time. We have also last year initiated the strategy of expansion of EMROK in other indications , as well as to other specialties and customers in India and we expect that going forward this should have a promising result from the initial indications we have. In our biotech space we have filed for marketing approval of DCGI to DCGI for Aspart.

Some of the other positives that we have

achieved in our organization in the last year has been

growth of various businesses across. So our emerging market business has grown by 10 % , India branded by 4 % , UK by 8 % and we have through business development and various partnership and collaboration developed a new business in biotech equivalent to \$30 million which will happen over the next 24 months.

We have had a very strong focus in this last year on operational excellence and cost reduction and if you see the financials our cost in this year is almost the same what it was in the previous year and what we have done is we have a team , a dedicated team to monitor various areas and to create value in terms of cost reduction initiatives and 3, 4 things , we are doing a large number of things .

But the few things that have had a significant impact has been a restructuring of manufacturing organizations from external to internal. We have initiated various energy cost reduction measures and we focused on wastage , identified where the wastage leakage is there in the organization , developed MIS , monitoring that, sensitizing the operational team on that and we have very significantly reduced wastage in the last year . And I think all that has helped in improving our profitability and EBITDA where we have seen growth of more than 60 % in this year.

In the middle of the year , we also had a QIP and we raised a INR1,000 crores . Thanks to the confidence of the investors , the investors have placed on us . So let me take a one minute break , I mean 15 seconds break . So how many of the investors over here would expect to hear something new that you've not heard before? Raise your hands .

Okay . So I hope I will not disappoint you and you will hear something new over here which you've not heard earlier . In case you haven't , you can directly get in touch with me. Okay . So
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our sales in the last year has grown by 5%, EBITDA has grown by 67 % and our EBITDA margin has expanded by 500 basis points from 9 % to 14 %.

Debt. So if you see the debt that we had , which is a net debt which was 882 in '22 has gone down to 64 in '25 and that's where we have reduced our external debt . Our cash available to us has improved and we are sitting on about INR600 crores of cash . And our equity last year which was INR3600 crores has gone to INR4600 crores . So financially overall we are in a much healthier situation this year than 1 year.

So looking ahead , where do we see the organization growing ? What is our areas of focus ? And how are we moving ahead and what is the approach that we have ? So in the next 3 years , there are 2 fundamental areas of growth . One is our diabetes biosimilar business , which includes insulin glargine and other products that we are developing and in the pipeline .

Second is our novel drug discovery antibiotic portfolio including ZAYNICH , Miquaf and EMROK . And as we look ahead in the 3 to 5 year period , we are developing even more insulin analogs GLP -1 analogs , like semaglutide and additional NC E molecules which are in the early stages of development , like Foviscu and Odrate which I mentioned earlier.

So since we are a research -driven company , no investor conference will end without a little bit of talk on science and research . And we take you through what is so unique , what is so special and as chairman said earlier , we have been the most successful drug discovery company in antibiotics over the last 15 -20 years , having a rich pipeline of 6 products in clinical development and now 2 products have been launched EMROK and Miquaf .

We have a team of 145 people in our research center , 50 of them are PhDs . We have been dedicated , focused , committed over this journey of 25 years and have developed end -to-end capabilities in this space , right from basic research to manufacturing to clinical trial and taking to the market. We have

6 programs that have been given QIDP status by FDA , which basically means that the molecules and the products we are developing are not a me -too kind of products , but are differentiated because they tackle and meet an unmet medical need.

This gives you a more comprehensive flavor of the various molecules that we have . What is the current state of development ? What is the indication and where it is targeted for use ? What kind of a market we are focusing on ? And how we are positioning the product in a competitive space ? So ZAYNICH is against resistant infections in the hospital which is UTI, pneumonias and various other resistant gram negative infections. It's a global launch . We have done a global trial doing in US, Europe, emerging markets, India . So it's a global market . And its positioning is a destination therapy for difficult to treat gram negative Acinetobacter and Pseudomonas . Foviscu which is a 4282 Phase 3 clinical trial is going on and it is also given in cUTI HABP/VABP . It's again a global trial , but here it is an empiric use . Empiric use is a first line treatment in gram negative organisms.

Odrate which I mentioned earlier is used in gram negative organisms for outpatient therapy . So instead of being in the hospital for 2 or 3 additional days just for the antibiotic here the patient

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can come back home or in the daycare center and get this injectable which is once a day treatment.

The other 2 products that we discussed is EMROK and Miquaf and they are in a gram positive space . EMROK is in MRSA infection . It is an injectable , as well as an oral and can be started in the hospital and then move on to outpatient care. It is used for skin infection . MRSA is also there in diabetic foot infection , which is a huge market . It is also there in orthopedics and bone infections osteomyelitis where the treatment has to be given for a longer period of time . And Miquaf is used in pneumonia , that's community acquired pneumonia and respiratory tract infection . It is in India , an emerging market we are focusing on initially . And this is where there is a huge resistance to macrolides like azithromycin and it is also quinolone sparing , which clinically has a significance .

So this is ZAYNICH. I think most of these aspects I have shared earlier . The only additional point I will make here is that we have an investigational break point of 64 milligrams , which is one of the highest break points of any antibiotic in or covering all the gram negative organisms over the last , I don't know , if I'm not wrong maybe last since the discovery of penicillin. So that

is the quality of the drug that is there . The other points I have mentioned earlier , so I'll not repeat. So we had a USFDA pre -IND meeting in May 2025 and we are planning to file in USFDA in the coming quarter . And generally the FDA takes about 9 to 12 months for approval . So we expect by middle of next year to be launching in the US . And in the subsequently in H2 of next year to file in various in Europe and emerging markets we will file in H2 of this year and launch in H2 of next year depending on the approval time.

How large is this market? So we have done extensive modeling of the disease , the indications where the resistant infections are and where a product can be positioned and used . And broadly there are about 2 million patients globally where we can use ZAYNICH , which includes about 158,000 patients in the US, 200,000 or so in Europe, a million patients in India and about 650,000 patients in China. This just gives you an idea of the kind of modeling we have done to arrive at the addressable target market.

So if I want to simplify this , I can ask you to look at the first column to the extreme left which shows 2.2 to 3 million cases and then the extreme right which shows 1 million cases and this is only for the India

a market , but basically just to show you the model how we have arrived at the model. So we look firstly the resistance level that is there . So in different organisms there are different resistance levels .

Then we look at based on that what are the drugs which are not treated by existing treatment options . And then finally , we arrive at the addressable market for ZAYNICH . So there are only two things we look at . One is resistant organisms and no available treatment . And once we do that, we get the addressable market specific for ZAYNICH . And this we have done for all the markets in the US, Europe, emerging markets, various markets.

So coming to India . We started at 2 million , addressable market is 1 million and from a potential point of view it is about INR17,000 crores. Now that depends on the market share we get and

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how we do in the market , but this is the potential target addressable market. Similarly that same exercise we have done in US and Europe and over there the TAM or the total addressable market is 7 billion , which is including 300,000 cases in US and Europe out of a total of 4.3 million hospitalized cases. So from 4.3 million after you filter down we come to the addressable market of 371,000 cases , and that represents from a value point of view a \$7 billion opportunity. So that was about ZAYNICH .

Now we go to Miquaf . So Miquaf has been introduced in the community acquired pneumonia and after 30 years there is a new antibiotic in the macrolide class which is there. If you remember earlier, we had started off with erythromycin in '52, roxithromycin in '87 and azithromycin came in 1988 and clarithromycin came in 1990. And since then there has been no other macrolide introduced and Wockhardt has introduced India discovered, India R&D, India developed clinical trial done in India , Maa product for the Indian market which we will now take outside India into the rest of the world after 35 years . And therefore it's a great pride that we feel to have done that not only for Miquaf, but the entire portfolio that we have developed over the last so many years.

So a little bit more detail on the unique specialness of this product. It's a broad spectrum ketolide for pneumonia and respiratory tract infection. Just to give you an idea today , Azithromycin resistance in pneumonia is greater than 65 % . So you can imagine the kind of unmet need which is there in treating pneumonia cases which are resistant to current antibiotics.

The other advantage is it is broad spectrum . So if you have a multiple organisms infection in pneumonia , which is gram positive , negative and atypical , Miquaf will be able to treat a wider spectrum of organisms and infection . And it has a superb penetration in the lung. The kind of penetration and the levels that are there in the lung are best in class , lung concentration which is there. Additionally , it has a convenience of a once a day therapy for a short period of time that is 3 days against conventional therapies which are there for 5 days and 7 days.

Phase 3 trials which we have done has shown excellent efficacy of 96.7 % and we have just launched in India on 27th May. Additionally we have got a QIDP approval from USFDA and Saudi Arabia which I mentioned earlier BMP. Maybe this is a little too much in detail . But broadly if you have a look at it and see the profile of Miquaf versus a competitive product , the green color is where we have a superiority over existing treatment , which are there for pneumonia .

And if you see various other products that are there have different levels of green , and the pink is where it is not as good and green denotes it is good. So like for Azithromycin in terms of resistance to macrolide it does not have. In terms of coverage to influenza it does not have. Prevalence of resistance very high. Lung concentration low. So like that for each of

them

clinically we have developed a competitive position for against every molecule and that's how we are positioning our product and also communicating to our clinical community . And we have done a lot of work in developing data with scientists in India and also internationally , collaborated with a lot of scientific community to establish this and have a lot of publications actually in the scientific journals that validate and all the data and information which we have.

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So there are about 367 million prescriptions of RTI , respiratory tract infection in India of that 62 million is in lower respiratory and 305 million is in upper. The antibiotic market size for respiratory tract infection is INR6500 crore at the current prevailing generic prices. What we are targeting is a market of 96 million prescriptions to the relevant doctor specialties which has an addressable market of INR10,800 crores at the Miqna pricing. So that was an update on ZAYNICH and Miqna and the various other NCE molecules that we are in development. Now I come to biotechnology . So, good unique position from a portfolio point of view , as well as having a competitive advantage . And we have integrated capabilities right from end -to-end. Over the years , we have also been in a biotech space for the last 25 to 30 years and we have developed R&D capabilities across technologies and with a focus on insulin and insulin analogs , we have four API blocks . We have drug product manufacturing sites in India too . And we also have a patented in-house device of both reusable pen and disposable pen. We are in India through our own organization and emerging market through both partners and distributors in more than 30 countries . And we have the entire portfolio of SKUs contributing to from vials, cartridges and pens . And this is an area where there is a limited competition of between 6 to 7 players . And in insulin and analog , in India , in emerging market it's about a \$1.5 billion market. In the pipeline we have Aspart R and Mix which is another \$700 million market. So our plan in diabetes is to have new products and access more and more emerging market and develop the biotech business and there is a huge opportunity over here. In addition to that , there has been one development in the last year . And Novo Nordisk , which is the largest insulin company in the world has decided to discontinue its disposable insulin pens and cartridges , so that they can have a larger focus on semaglutide and other GLP -1 analogs , where their capacity is not able to meet the demand . And as a result of that , that space opens up for existing players in insulin market. The equivalent market in India is about INR450 crores and in emerging market is another \$157 million. We are well positioned in this space to capitalize on this opportunity and we are also ramping up our capacities and in fact doubling -- want to double our capacities over the next 24 months and put up new facility to take advantage of various opportunities that we are seeing. So these are the status of the products we have which are not yet launched and the development that we have done. So Aspart R is done , 30-70 analytical similarity , PK/PD is to be done and Lispro is further up in the development.

So I will end by a summary slide that most of you and maybe some of you who haven't have invested in a company that has multiple levers of growth , and we expect that to continue if not accelerate. Driven by the strong performance in the last year and the continued momentum that is there , which will get further accelerated by our novel antibiotic portfolio , as more and more products come to the market and access larger markets in addition to India , various emerging markets and the U.S. and Europe.

ZAYNICH, which is a breakthrough in innovation has an addressable market of about 9

billion

globally and Miqna has an addressable market in India of INR10,800 crores. Our diabetes portfolio in the emerging market which includes Insulin Glargine and Aspart has an opportunity

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of about \$3 billion . As I mentioned we are doubling our capacity in the next 24 to 36 months and that would help tap into the growing demand and we expect to double the business in the next 3 years. Okay . Thank you very much for patiently listening to us . And if there are any questions we would be happy to answer them.

Moderator: Thank you very much , sir. We will now begin the question -and-answer session. We'll take questions from investors present at the venue. Investors present at the venue may please raise their hands. Our on-ground team will pass the microphone to you. Meanwhile those who have joined us online may please type their questions in the chat box. In the interest of time kindly limit your questions to the company's future outlook or growth strategy. Operational or routine

worries can be addressed separately by the Wockhardt team. The investors present at the venue are requested to introduce themselves and proceed with their questions.

Ladies and gentlemen we'll start with the text questions and the questions are, the first question is what is the probability of ZAYNICH getting approval from USFDA in FY '26-'27? Do you find any hurdle in approval process? How long will ZAYNICH India approval take since we have filed in March 2025? And what will be the timeline for commercial production and market availability?

Habil Khorakiwala: You have two questions on ZAYNICH. One is regarding India and one is regarding the US. We expect ZAYNICH approval sometime coming at the beginning of next year and we should be in Indian market in the middle of next year, another 3, 4 months thereafter. And we see a high probability of this because we applied in March, normally they should take 6 to 9 months for approval.

As far as the manufacturing is concerned for India, it is all it is ready and we are making adequate quantity to meet the demand for the product. Coming to US, there is a very high probability or certainty I would say that we should be able to market the product sometime in '26-'27. Yes.

Unknown Analyst: Yes. Three question. Like, how do we intend to market Zaynich is by the company. And just throw some granular details on that side of it sir, if you can. Thank you.

Habil Khorakiwala: Let me understand your question properly.

Unknown Analyst: Sir, how do you intend to market ZAYNICH in US, like a tie up with someone or someone else will be doing, who would be manufacturing some granular details.

Habil Khorakiwala: Yes, okay. Fine. As far as India is concerned, we would be doing it ourselves. We have already built up a top leadership team and they have come from the organization who have the experience of launching new drugs in India. Now as far as the US is concerned, we are simultaneously operating on 2 options to optimize our value proposition.

One option, we are seriously thinking to create our own organization for which we are in the process of recruiting top team and also have consultant for various aspects because ZAYNICH is only prescribed and used in large tertiary care hospital and it is not a mass market product. So from that perspective, number of resources you require to cover is very limited. That is one option.

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Second option, we are simultaneously looking to out license the product with some of the big pharma or any suitor. Our objective basically there is if we get a good valuation for what we believe ZAYNICH value is, we may consider that as an option. But if you don't see that kind of valuation coming, we will do it ourselves.

Hansal Thakkar: Good morning Dr. Khorakiwala. My name is Hansal Thakkar and I'm represented here by all

the investors. So first of all a heartiest congratulations. I think people don't understand the gravity of what you, your scientists, your entire team have created.

We have been tracking this development from the very first compassionate case of the Nepalese patient. And since then, we have had nothing but admiration for you and your team, which, sorry I can't avoid it, but is the true odyssey of courage. I speak on behalf of all the investors when I congratulate you again when the government recognizes Nafithromycin as our poster boy to biotechnology.

And at this moment, I don't want to ask any questions. But apart from the financial aspect, I think my speech is a little more emotional. So I would thank you and I would request everyone here to give us a standing ovation. This is nothing short of a miracle. So, congratulations, sir. I'll pass the mic on to someone else who might want to ask a question.

Habil Khorakiwala: Thank you very much for your appreciation and more importantly recognizing what is being done. So our achievement or what we have done is absolutely a major breakthrough from Indian context and that is not yet fully be recognized. Thank you very much for recognizing that and wonderful for good words.

Bharat Sheth: Hello, sir. My name is Bharat Sheth. I have this question on the amount of fundraising the company has done via the equity dilution, as well as the raising of debt. So the pipeline of the products which we have mentioned apart from ZAYNICH. Are we any such in the further equity dilution funds, R&D or the marketing part of it by way of debt or by way of debt? So can you just give us the picture, I mean, the probability of the debt going out and the equity dilution going ahead. Thank you so much.

Habil Khorakiwala: We just raised about INR1,000 crores a few months back and we are having -- still sitting on some cash basically. Also you have seen our EBITDA performance has improved. This year it will be much better. And I think subsequent here with the launch of NCE, we will have much better cash flow. So we believe mostly this cash flow coming in would be more than adequate for research. We have been spending the last few years on new drug discovery about USD 200-

250 million . Probably we may continue to spend that or little more. So I think it would probably meet our requirement basically.

Unknown Analyst: Thank you so much.

Unknown Analyst: Hello , I am Dhruvesh Sanghvi. If you can understand the journey that a new product typically takes. Let's say like India because we will have ZAYNICH coming soon in India. So is it like a 3, 4 year journey where it goes up and it takes a long time. Or is it like a couple of quarters and reach to the target audience , make them aware ? Because as you said it's not a product , but a specific hospital product. So if you can throw some light. Yes. Thank you.

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Habil Khorakiwala: See one of the objectives of launching. I'll give you our approach for Mignaf and then ZAYNICH . In terms Mignaf, what we are doing in the first 6 months is only promote to chest specialist so that the specialist endorses the product and it is a chest specialist who get most of the resistance cases because the patient has gone to other doctors and then they go with difficult cases to chest specialist . And about one third of the patient chest specialist receives are untreatable by any oral treatment. So normally they refer it to the hospital for injectable treatment and admit to hospital.

So therefore we said, we will focus on chest specialist for 6 months and then expand the coverage of consulting physician and ENT and pediatrician. But we will be restricting our promotion for next 3 to 4 years only to specialists and we will not plan to go to a GP . For two reasons. One is it is better that a

new drug gets a right endorsement from the specialist group as a whole , so that it has a much more long sustainable life.

And secondly , if we are looking at the price point which we are having , so in terms of treatment costs it is very cost effective . Because when you have a failure you need further treatment , you have to go to hospital and the whole treatment is high . But from that point of view when the cost of a medicine is high , say INR3,000 per treatment of 3 days, but total treatment cost is very reasonable.

Now coming to ZAYNICH . ZAYNICH is generally used in the -- see first two things we have done . We introduced EMROK and we didn't have a good experience because we were new to it , so we went through a major learning curve internally . And for Mignaf and ZAYNICH, we have built the organization and literally our top team we have got it from multinational companies who have the experience of launching new drugs.

So the same thing we have done for ZAYNICH. And from day one we would be going very quickly to almost all doctors ZAYNICH by multi -channel approach , because we are having a very large number of conferences where a scientific element will be shared , plus we are creating a slightly innovative business team where we have populated our medical doctors to a fairly high ratio 1:4, 1:5 to representative . Mainly because you have to communicate the scientific aspect of the molecule and a qualified medical doctor can do a much better job. So we don't expect years to get into a doctor to appreciate , we expect doctor to appreciate this molecule very short time.

Moderator: Thank you sir. We'll take the next text question and the questions are manufacturing readiness for US market . Could you please confirm which facility is planned to manufacture WCK 5222 for US market ? And whether the site has cleared or is ready for USFDA pre -approval inspection ? And the next one is commercialization strategy , has Wockhardt entered into or is in advanced discussion regarding the commercialization or outlicensing partnership for WCK 5222 in the US or EU ? If not , then when is such development expected?

Habil Khorakiwala: Very good question. There are 3, 4 aspects about 5222. One is manufacturing. The whole manufacturing is being done in Europe both for API , as well as formulated product . And we are already completed initial batches required for filing long back . And I think we would be well ready to make whatever initial our demand would be there for manufacturing from US . And Wockhardt Limited

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their existing people , existing capacity , they -- all facilities are approved by USFDA . So that manufacturing is totally we have the risk that -- that is one. The second element of your question. Second question was your commercialization. I did explain earlier the commercialization and as we have more details , I think I'll let you know. As far as the licensing is concerned , licensing is moving quite all right. At the same time , our own organization creation is moving space . So we might in next few months have a CEO anywhere for the organization . Because if I have to get more value for my proposition , I have to be very serious of creating my own organization as an

alternative.

So that -- it's not that for people come and give me a rate . But if I have expectation higher than that, I have to say that I have an alternative available if I believe this is a value we will . So that is a very deliberate part of a strategy. It may cost a little more extra , but that is what we are doing . Because of value proposition , looking at the molecule has to be very significant according to us because these are the drug available once in a while.

Normally the industry trend is whatever recent antibiotic sales purchases happen t hey put little , within that range they try to work and everybody works on the same basis.

So I have to get

somebody out of the thinking process that -- so that is what our strategy as far as outlicensing or manufacturing is concerned . And the last query wa s?

Unknown Analyst: No, I think that's it, yes.

Habil Khorakiwala: Does this cover ? Does this cover your questions?

Moderator: Certainly , sir. I'll take the next text question , and it says firstly many congratulations to Dr.

Khorakiwala and the entire team of Wockhardt for their stupendous achievement on cracking the lifesaver antibiotic innovations that will make the whole India proud. I have been an investor with significant quantities of the shares since last 1.5 years and would be gra teful if the below queries get addressed. Normally doctors prefer to use the antibiotics that they are familiar with . And how much time ZAYNICH may take to build this awareness and familiarity.

Also will there be any impact on the potential pricing of the drug that is USD8,000 to USD 10,000 per patient due to Trump's policies. Does ZAYNICH can also replace polymyxin which is currently last resort medication if a Carba penem fail, but they are also neurotoxic. The addressable market figures are on per annum basis.

Habil Khorakiwala: Addressable market is ?

Moderator: The addressable market figures are on per annum basis.

Moderator: Annually.

Habil Khorakiwala: These are annually th o se presentation numbers everywhere. Coming to your earlier question.

Moderator: How much time ZAYNICH may take to build the -- this awareness and familiarity.

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Habil Khorakiwala: In India , okay , fine. I think our approach for ZAYNICH is, the primary market of ZAYNICH is with the specialist that is intensivist in the ICUs and disease specialist to recommend to use a drug. So that is the target market we are trying to address very immediately , by various method to meeting , conferences , involving a large number of medical scientists there .

And we hope that we access this market within first 6 to 9 months and even if we get 20 -30,000 patients we are looking at something like INR200 crore , INR 300 crore , INR 400 crore sales. So that is our aim . But I think in 3 years we should be looking at 80 -100,000 patient potential for ZAYNICH .

Moderator: Thank you sir. We'll take the next text question , and the question is , do you have the manufacturing capacity to cater to all markets for ZAYNICH ?

Habil Khorakiwala: So western market , I already mentioned that we have the capacity , we have checked with the suppliers , manufacturers and they have adequate capacity to meet western market. As far as India and emerging market is concerned , we are developing in -house capacity in India and third - party manufacturing we are doing for formulation. API we are manufacturing here for all our products . And for next 2-3 years , we today have the capacity and we will build up capacity as a demand increases and make sure tha t there are no shortage of capacity.

Moderator: Thank you , sir. Ladies and gentlemen in the interest of time this will be the last text question , which is sir what kind of outlicensing and royalty earnings are you expecting for ZAYNICH over the next 5 years?

Murtaza Khorakiwala : Out licensing income and royalty on sale you expect over the next 5 years?

Habil Khorakiwala: A very difficult question to answer that basically. So one could -- the idea was that we gave you the overall potential addressable m arket. So obviously , we will be doing our best possible job . As an organization , we would also be learning with market dynamics is taking place. So what we will do to get optimum value is we will have a good team of people , in terms of commercial and medic al and scientific .

And I must tell you , our scientific team which has discovered the drug and what unique thing happening with us and our success in the past and likely to be in the f

uture , is there are about 15

disciplines required to discover a new drug which are independently like chemistry, biology, pharmacology, microbiology, toxicology , like that 15 discipline is required.

Now our team -- over the years we have created a team which interact with them on a regular basis. So they work as a team and that is why the innovation comes. It's not a silos , but it is

working as a team . And I think we have extended that model to our business also now with the scientific team.

So they are totally in sync , because scientists know much more of the molecule because they have lived with the molecule for last 10 -15 years. So whatever they plan to transfer the knowledge to medical or commercial people it will not be same level. So we are keeping them as a part o f overall business development , so that our scientific profile , whether in India or US

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or anywhere will be second to none , and I believe these are important elements for the success of the drug.

Moderator: Thank you very much sir for answering all the ques tions. We will now close the Q&A session.

Thank you for your understanding. I would now like to hand the conference over back to Dr.

Murtaza Khorakiwala for his closing comments. Thank you and over to you , sir.

Murtaza Khorakiwala: So, I think I would like to thank all the investors and all those who have come here for our conference and participating in the event. Thank you all for coming here. I hope as I said you would have learned something new about the company and you take back with you . And look forw ard to your continued partnership and support in our journey. We have tall ambitions and aspirations . And I am sure that your support will be very important in achieving those objectives.

Thank you again for your time.

Moderator: Thank you members of the management. Investors present at the venue may continue with the conversations with the Wockhardt management. On behalf of Wockhardt Limited that concludes today's conference. Thank you for joining us and you may now exit the meeti ng. Thank you , sir. Have a great day.

Call: Jun 2026

Ref. No.: WOCK/SEC/SE/2026 -27/017 11th June, 2026

BSE Limited
Corporate Relations Department
P J Towers
Dalal Street
Mumbai - 400 001
Scrip Code: 532300 National Stock Exchange of India Limited
Exchange Plaza
Bandra Kurla Complex
Bandra (E)
Mumbai - 400 051
NSE Symbol: WOCKPHARMA

Dear Sir/ Madam,

Subject: Disclosure under Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended –Transcript of Investor meeting/ call .

Pursuant to Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended and in continuation to our letter bearing reference no. R ef. No.: WOCK/SEC/SE/2025 -26/015 dated 1st June, 2026 , please find enclosed the transcript of the Investor Meeting held on Thursday, 4th June , 2026 , for your information and records.

The said Transcript will also be uploaded on the Company's website and can be accessed through the following link:

<https://www.wockhardt.com/wp-content/uploads/2026/06/wockhardt-investor-meet-jun-04-2026.pdf>

Kindly take the same on your records.

Thanking you,

For Wockhardt Limited

Rashmi Mamtura
Company Secretary

Encls: A/a

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“Wockhardt Limited
Investor Conference Meet/ Call”
June 04, 2026

MANAGEMENT : DR. HABIL KHORAKIWALA – FOUNDER CHAIRMAN –
WOCKHARDT LIMITED
DR. HUZAIFA KHORAKIWALA – EXECUTIVE DIRECTOR
– WOCKHARDT LIMITED
DR. MURTAZA KHORAKIWALA – MANAGING
DIRECTOR – WOCKHARDT LIMITED
MS. ZAHABIYA KHORAKIWALA – DIRECTOR –
WOCKHARDT LIMITED
DR. MAHESH PATEL – CHIEF SCIENTIFIC MENTOR –
WOCKHARDT LIMITED
DR. SACHIN BHAGWAT – CHIEF SCIENTIFIC OFFICER –
WOCKHARDT LIMITED

MS. ANNAPURNA DAS – PRESIDENT INDIA BUSINESS
AND NCE EMERGING MARKETS – WOCKHARDT
LIMITED

MR. WILLIAM MCNEY– CHIEF COMMERCIAL
OFFICER – NOVEL ANTIBIOTICS BUSINESS

DR. DENNIS DERUELLE – CHIEF MEDICAL OFFICER –
US NOVEL ANTIBIOTICS BUSINESS

MR. LEO YASINSKI – VICE PRESIDENT , MARKET
ACCESS NCE BUSINESS

MS. SANDY ESTRADA – SENIOR NATIONAL DIRECTOR ,
FIELD MEDICAL AND MEDICAL SCIENCE LIAISON

Dilini Quadros : Ladies and gentlemen, good evening, and a very warm welcome to Wockhardt's Investor
Conference . I'm Dilini Quadros from Wockhardt, and I will be your host this evening. We are

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delighted to have you all join us here today, both at the NSE Exchange Plaza and virtually from
across India and overseas. Thank you for taking time out to be with us as we share some of the
exciting developments and milestones in the Wockhardt journey.

I would like to invite the Chairman and the top leadership team of Wockhardt to join us here on
the dais. Dr. Habil Khorakiwala, Founder Chairman; Dr. Huzaifa Khorakiwala, Executive
Director; Dr. Murtaza Khorakiwala, Managing Director; Ms. Zahabiya Khorakiwala, Director;
Dr. Mahesh Patel, Chief Scientific Mentor; Dr. Sachin Bhagwat, Chief Scientific Officer; and
Ms. Annapurna Das, President India Business and NCE Emerging Markets.

We are also pleased to be joined virtually by members of our US core team: Mr. William McNeely,
Chief Commercial Officer, Novel Antibiotics Business; Dr. Dennis Deruelle, Chief Medical
Officer, US Novel Antibiotics Business; Mr. Leo Yasinski, Vice President, Market Access NCE
Business; and Ms. Sandy Estrada, Senior National Director, Field Medical and Medical Science
Liaison. A warm welcome to all of you.

Today's session will begin with a presentation by Dr. Murtaza Khorakiwala, giving an overview
of the organization, followed by presentations from Ms. Zahabiya Khorakiwala, who will focus
on our plans and strategy for Zaynich in the USA, and Ms. Annapurna Das, who will focus on
our strategy for emerging markets. We will also be sharing a few short films during the course
of this event.

Following the presentation, we will open the floor for a question -and-answer session for both
in-person and virtual participants. And for those of us joining online, the lines will remain in
listen -only mode during the presentation. Questions may be submitted through the chat function
and will be addressed during the Q&A segment. With that, it is my pleasure to invite Dr. Murtaza
Khorakiwala, Managing Director of Wockhardt Limited, to begin the presentation. Over to you,
Dr. Murtaza.

Murtaza Khorakiwala: Good afternoon, ladies and gentlemen. A very warm welcome to you on behalf of the Wockhardt
board and the senior leadership team. And thank you for joining us today. A big thank you to
our shareholders, analysts, lenders, employees, healthcare professionals, and business partners
for your continued trust and patience.

Wockhardt's journey of transformation has begun. We have faced challenges, made difficult
decisions, sharpened our focus, and invested with conviction in areas where we believe we can
create long -term value. We have emerged into a stronger, more focused, and innovation -led
healthcare group. I am pleased to say that this strategy is delivering results. Wockhardt, as you
know, in the last few days, is now the first Indian company to have an Indian research product
approved with the US FDA, Zaynich . And more importantly, we are now entering a phase that
is even more exciting.

Next slide, please. Next slide. Next slide. Wockhardt is a global research -driven multinational
organization. Fundamentally, we have three pillars and platforms, verticals of our business. One
is our pharmaceutical business, which to day comprises of about 75% of our business. Second is
our biosimilar and biotech business, and third is our novel antibiotic business. And in a very
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interesting way, this creates a synergy against different verticals in terms of cross -leveraging the
advantages that we have in one vertical against the other. Plus, they span a different time horizon,
cash flow cycle, and that gives a sustainability and stability to the entire operating model. We

have in-house 11 manufacturing facilities and two R&D centers.

You would be pleased to know that this year we have delivered good financial results. Our top line is at INR3,373 crores, with an EBITDA of about INR630 crore

es, and EBITDA growth of

about 51%, and our profitability before tax at INR238 crores. From a liquidity point of view, we are sitting on a cash equivalent of INR662 crores, and our net debt-to-equity ratio is 0.1.

Of the various verticals that we have, the specialty business that is there, which is consisting of biosimilars and the novel antibiotic, contribute 23% of the business. And the biosimilar biotech business in the current year has grown by 27%, during which, as a result of productivity and production improvement, we have scaled up our production of human insulin by 2x and Glargine by 1.5x.

Looking from a regional point of view, you would see that all our businesses are growing and performing in a healthy way. Our emerging market business, which today is 28% of our turnover, is growing handsomely at 35%, aided by strategic partnerships that we have been able to arrive at over the last 12 to 18 months in Brazil, Thailand, Algeria, and Malaysia. Our UK, which comprises of 39% of our total business, is growing by 13% and is focused on specialty injectables. Our India business, driven by our innovative portfolio of Emrok, Miqna, and regenerative medicine, and Ireland is supported by our liquids and creams product portfolio. Just to give you a perspective of our focus on profitability, which has been there for the last three years. Three years back, our EBITDA margin was about 5.4%, which has now increased to 18.6%. And we have had a very clear objective within the organization to focus on profit by maximization of product portfolio with high margin.

We have exited the loss-making US generic business and by various initiatives in terms of supply chain optimization. Supporting our focus on profitability has been also a very clear focus on cost management via various initiatives of operational excellence, and where we have implemented almost 50 projects across the organization in different areas to manage our costs proactively.

In addition to that, we have also restructured some of the supply chain and manufacturing from an efficiency point of view. And during the last year, we have implemented S/4HANA, which is the new version of HANA, which is a cloud version, and we are working on embedding various AI-related initiatives within the organization.

So, looking ahead, having achieved a good profitability over the last three years, looking ahead, let me take a couple of minutes to share with you what I believe are the key drivers for the organization going forward. So, the first platform that is there is the novel antibiotic platform, where with the launch of Zaynich globally in US, India in the current year, and then Europe and emerging market in the subsequent years will be there.

Along with that, we are developing the Emrok franchise in India and Miqna in terms of scaling the business that we have in India. Supporting our growth initiatives is our focus, continued

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focus on operational excellence, where we will be looking at building talent and developing a core leadership team with new capabilities and new talent where the expertise is required as we are entering into innovative space and expanding our horizons into new areas, new markets. We are also looking, as I said earlier, how we can use AI more efficiently, more effectively to reduce time and reduce cost, and looking at how we can make this sustainable and scalable. We have launched few initiatives in the last year, but we would like to scale it to larger aspects of organization and also look at how we can develop sustainability. On biotech and pharma, I will go a little bit more in detail.

So, the other vertical of growth, platform of growth we have is the diabetes biosimilar franchise. And in biosimilars, as you would have noticed, there are a large number of players now in biosimilar space. Like in innovation where we focus i

not only one area, that is antibiotic, similarly

in biosimilars also, we are focused only in one area, and that is diabetes.

Diabetes in India and emerging market roughly is around \$7 billion to \$8 billion market. It has limited competition, about six, seven players, unlike monoclonal antibodies which have 15, 20 players. Here we are looking at developing, or rather have already developed, a completely end-to-end integrated capabilities right from research to API to manufacturing to sales and marketing.

And we already registered in 30 emerging markets, and the business, as you saw earlier, has done well, growing at about 35%. So, we have two products commercialized and five more products in the pipeline, which include Aspart, RN 30/70, Degludec, Degludec Aspart, and Semaglutide.

The fourth pillar of our growth is our pharma core business, which is about 75% of our business

today, and where we have substantial presence in India, UK, and Ireland. India is mainly driven by our portfolio in diabetes, orthopedics, and pain, and the new therapies that are coming up in the regenerative medicine. UK and Pinewood, both we have a leadership position in the market in terms of being in the top five.

In UK, in the covered market, we have 18% market share. We are there in most of the segments in the UK market, whether it is hospital or pharmacy or export or OTC or even branded generic. And generally, we are looking at developing a more differentiated R&D portfolio within the pharma space from a competitive point of view. So, this is the fourth area of growth that we have.

Just to take you back where we are today and where we were eight years back. In the first five years from 2018 and '23, we focused on liquidity management. Literally, we were focusing on surviving because of strong liquidity pressure, and we did various things in terms of raising money at the QIP, deleveraging the organization, debt restructuring, squeezing our working capital, and all that put together helped in creating a liquidity and we were able to get out of that. And over the last three years, the management has been focused in driving profitability, and as you saw, we have worked on that and improved our EBITDA from 5% to 19%. And the next journey that we have is of scaling our business and growing our business. And I think that's a

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very exciting journey, and that will basically, as I also said earlier, will include creating a large global Zaynich business, acceleration of our biotech business, which is a very large opportunity in terms of the market that we have, in terms of the portfolio that we can create.

So, that is another great driver of growth. And we will maintain our cost competitiveness and operational excellence initiatives, and in fact, even scale that further, which will form a very sustainable platform of growth going forward.

For many years, investors asked us when the investment in research, biotech, and innovation would begin to translate into growth. We believe that moment has arrived. With Zaynich, with our expanding biotech platform, with our strong core business, we are entering a new phase of Wockhardt's evolution. We look forward to sharing that journey with all of you. Thank you once again for your support and trust.

Thank you. There is now a short film which I would like to invite you to have a look at, and following that, Zahabiya will take us through the US launch strategy for the commercial business.

[Video Presentation 0:18:47]

Zahabiya Khorakiwala : Hi, good afternoon. It's my absolute pleasure to welcome you all today and to meet you. It's a very proud moment for all of us in Wockhardt, and I'm sure for each one of you and every Indian here today. I'll just quickly introduce myself because we haven't met before. I'm Zahabiya Khorakiwala, I'm the Managing Director for Wockhardt Hospitals for

the last 15 years. I will be

leading the US operation with the US team for the launch of Zaynich and the business going forward.

Over the last six to eight months, we've done a few things to ready ourselves for launch preparation. The first is, of course, to build a really deep, strong top leadership team, which I will, quickly allow them to introduce themselves a little bit later. I have also really kind of sunk myself really deep in the science to see how it differentiates itself in the marketplace vis-à-vis competition.

And I've also met a lot of customers, hospital systems, to really understand and formulate a strategy which will be ready for execution in the next six to nine months. So, I'm just quickly going to take you through a high-level view of how we're going to be going about our view of how Zaynich is placed and positioned in the US, and also a little bit of, the market size, the dynamics, etc.

Next slide, please. So, Zaynich has been approved by the US FDA. It is the first of its kind, it's monumental and historic. But beyond the approval, I want to really kind of, what's really important beyond the approval is what the FDA has called out in their approval. That it really covers the pathogen. So whilst we have received an approval to cUTI, but there is a mention of covering enterobacteriales and pseudomonas, which are the major gram-negative infections caused today in the US or globally.

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It's also made a mention of the breakpoints, and the breakpoints that we have been granted, these

cover almost 95% of carbapenem-resistant cases in the US. So, it is extremely important to know that yes, we have an approval, but we also have a mention of the entire coverage of gram-negative pathogens and almost, and the breakpoints which cover most resistance in the US. And this is extremely important in the US market where clinicians are extremely knowledgeable, they understand the nuances of how a drug can work in a patient.

Next slide, please. This is just a little bit of macro market. There are about 1.2 million gram-negative hospital infections in the United States. The indication for which we have approval, which is cUTI, is about 600,000, it's the largest indication, followed by HAP/VAP, complicated intra-abdominal infections, which is about 290,000, bloodstream infections another 60,000, and then another, all the other combination of infections, another 165,000. So, this is the high-level market size in the United States.

Next slide, please. This is specifically for cUTI in the US. So, when a patient, I'll just explain this a little bit, the first point. When a patient comes into the hospital setting, the cultures are taken to identify the pathogen. But prior to identifying the pathogen, the clinician has to start treatment.

And this treatment that is started empirically in high-burden settings, there's a 40% failure rate. And this really points to the fact that the infections are complicated, they are multi-microbial, they are also multi-mechanisms of mutation. And therefore, empirically, the choice of treatment is often not the right choice, and which often has to change post the susceptibility testing coming in.

About 15% to 25% of them are multi-pathogen infections, so it requires different drugs, or not a single drug is able to address the issue completely. The hospitalization risk in elderly patients, which is 65 and plus, is 3x to 5x higher. And almost 20% to 25% of infections recur and result in re-hospitalization.

This is very important in the US setting because the re are tremendously high, it's a high burden of healthcare costs when you have a re-admission of a patient. And the other drugs that are available today do not have a complete spectrum of pathogen coverage unlike Zaynich, as well as a complete coverage of resistant mechanisms, both of which Zaynich address

es completely,

almost completely.

Next slide, please. This is a little bit of science. I know the scientific team is here, but I think it's important to kind of point out the science a little bit because it's extremely differentiated. It is not another novel antibiotic that is in the marketplace.

Unlike today's options of beta-lactam, beta-lactamase inhibition antibiotics, which primarily look at inhibiting the lactamases that are produced by the bacteria, Zaynich, because of its composition of cefepime and zidebactam, zidebactam essentially is hydrolyzing the bacterial cell wall and making the bacteria dysfunctional.

Because of that, the antibiotic, the combining it with cefepime creates a very, very rapid antibacterial effect and rapid killing of the bacteria. This is very different from the current

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approaches which are limited. And this synergy is what really differentiates Zaynich in the marketplace.

Next slide, please. And, you know, we've seen this translating into the clinical setting. If you look at our Phase 3 clinical trial, which was against Meropenem, we had a composite cure rate of 96.8%. And the clinical and micro cure rate was about 89%, 90%. This was 20% higher than Meropenem. And the differentiation here is that our micro cure rate was extremely high.

And what this means is that when you have a high micro cure rate, it means you've eradicated the bacteria. Not just that the patient has gone home, but on a culture, you have actually eradicated the bacteria, and therefore the likelihood of readmission into the hospital is likely to come down sharply.

The other study that we did in India was a clinical efficacy Meropenem resistance study with about 60-odd patients. Here also, we had a 90% efficacy. The interesting thing here is that we covered the range of pathogens. So, we had enterobacterales, pseudomonas, Acinetobacter. We also covered the indication spectrum. So, we had about 15, 16 HAP/VAP patients, we had intra-abdominal patients and BSI patients. So, it covered both pathogens as well as cross-indications, and we had these efficacies.

And the last one was really, of course, the compassionate use. I think that has been a very satisfying for us that we were able to save patients that had no option. There were 85 patients across India, the United States, Malaysia, and France. And again, these were when all other options of treatment had failed, and this spoke pretty highly in terms of even resistant pathogens that we were able to treat in these patients.

Next slide. This is the leadership team, actually, that we've onboarded. We have a couple more

on the way joining us soon. The idea here is that we're going to onboard the top leadership team to drive really the business strategy, you know, minutely kind of be responsible for the execution of the strategy. They're all from industry, I will quickly ask them to introduce themselves one at a time.

We can start with William, if you could introduce yourself and then followed by the rest.

William McNay: Certainly. Thank you, Zahabiya. It's a pleasure to be here. As Zahabiya said, my name is Bill McNay, and I'm the Chief Commercial Officer here in the US for the NCE business. I've spent more than the past 30 years in the US pharmaceutical and biotech space driving commercial growth, launch, organizational performance across both infectious disease, oncology, pain, and many other specialty therapeutic areas.

I have significant experience in and around the hospital medicine space, including complex acute care markets and introduction of innovative anti-infectives and specialty markets into these channels. Over the past few decades, I've served in senior leadership and general management roles, including sales, marketing, market access operations, and corporate transformation functions that have delivered measurable business impact.

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Recent and most relevant experience, I think, is my time at Shionogi. I served on the US leadership team and played a key role in the commercial development and launch of Fetroja or cefiderocol, a first-in-class hospital anti-infective that Zaynich will be competing directly with. Thank you very much, and I turn it over to you, Zahabiya.

Zahabiya Khorakiwala: Dr. Dennis, would you like to go next?

Dennis Deruelle: Thank you, Zahabiya. I'm Dr. Dennis Deruelle, and I am still a practicing physician at a teaching hospital in Boston. My background at UNC Chapel Hill as Chief Resident, I was in infectious disease fellow. Also, I've had over 20 years as a physician executive leading hospital medicine and critical care over thousands of physicians, over 300 health hospitals and major health systems in the US. I've also had a consulting company, Operion Strategies, and I've been critical in advising companies like Cubist, The Medicines Company, Salix, Cepheid on their testing for COVID, and Gilead in their treatment for COVID. Thank you.

Zahabiya Khorakiwala: Sandy?

Sandy Estrada: Hi, I'm Sandy Estrada, and I am happy to be here today. My role is leading the field medical team, including the MSLs for Wockhardt US. I have over 20 years of experience in the infectious disease space, branching over both clinical practice and the pharmaceutical industry, leading medical affairs teams, launching new antimicrobials at multiple different companies.

And now I am excited to be here and to be part of the team bringing Zaynich to the United States where now more than ever we need this unique antibiotic to help combat the ever-increasing problem of multi-drug resistant gram-negative infections. Thank you, everyone.

Zahabiya Khorakiwala: Thank you. Leo?

Leo Yasinski: Thank you, Zahabiya. Good afternoon, everyone. Leo Yasinski. I will be running the market access here in the United States, I'll be Vice President of Market Access. My career spans for the last 25-plus years, starting all in antibiotics and remaining in antibiotics, starting with Wyeth with Zosyn and Tygacil, moving on to Cubist and Merck where I launched daptomycin, Zerbaxa, Sivextro, and Difcid.

And then finally was at Tetrphase and ran market access there for eravacycline, which was acquired then by La Jolla and Innoviva with the launch of Xerava and Septabipro. My strengths will be in the strategies of the OPAT market and IDN space with contracting, and I'm excited to be on this team. Thank you.

Zahabiya Khorakiwala: Thanks, Leo. If we could just have the next slide. Yeah. Like I was mentioning about really the broad strategy, which is that we're building out a strong leadership team that has deep domain knowledge, anti-infective knowledge. They have been part of organizations that have launched and led commercialization in the antibiotic space. And we will be operationally light.

I think what that means is that we're partnering with a commercialization partner because the US market is pretty complex in the many things that need to be done to launch an antibiotic and to start a new business. And so the entire operating activity of the US business will be, we have

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a commercial partner who will be taking care of some of those legalities, nuances within that. But the strategy and execution responsibility will be really driven by the leadership team that we are onboarding ourselves.

There are -- I've mentioned a few things. Market access is extremely important in the US, and this really means getting access into the hospitals. The idea and the strategy is to get maximum hospital penetration quickly and to really get a deep clinical adoption. So, there are centers and there are hospital systems, academic centers, etcetera, which have strong clinical buy-in, they have high levels of resist

ance, and they need alternatives to current treatment. So, we will try to

go deep in institutes where these needs and segment the market accordingly.

We will also focus very deeply on clinical advocacy through ad boards, through peer-to-peer interactions, relationships. The clinical piece is extremely important. It is a drug of the medical team, so to speak. On the logistics side, we'd actually put in place, again, this is outsourced, we put in place a partnership, a distribution model, a 3PL distribution model.

And I think from an economic value, which the healthcare systems and the hospitals are very, is very important for them, is really to showcase the economic benefit of the drug. And what that means is that if Zaynich is able to bring down readmission rates, if they're able to see that patients' length of stay is reduced and it is beneficial for them, the outcomes also justify the economic rationale. That is a study and that is data that we're going to be collecting and showcasing on an ongoing basis to really be able to present the economic value of Zaynich in the hospital setting.

We're deploying the three large -- I mean, there are three main categories of team that we're putting in place; the MSL and the medical team, the sales team, and the market access team. These are the three teams that will be working together to drive each of these areas in the US. And we've also got a US FDA manufacturing site is approved in Europe from where we will be supplying the drug.

Yeah, so this really sums up. I'm happy to take any questions, we will take questions later. But this was just a little bit about how we're planning to go about the launch of Zaynich in the US. Thank you for coming, and I look forward to interacting with you all.

I just like to welcome Annapurna Das. She is our President for India and NCE Emerging Markets to take it from here.

Annapurna Das: Respected Chairman sir, Wockhardt leadership team, and respected audience for coming here. You heard Zahabiya talking about that how in US as Wockhardt we are bolstering our presence and how we are building a very integrated launch mechanism which builds for an innovative asset like Zaynich.

Let me introduce myself. My name is Annapurna Das, I am President for India and NCE Emerging Markets. I come with 25-plus years of experience in healthcare and in pharma. And when I met Dr. Habil Khorakiwala and his vision for Zaynich and NCE and for Wockhardt, I think I had to make this choice to come and be part of the Wockhardt family. And nothing more, you know, you can see the emotion in my voice that I feel extremely proud and privileged to be Wockhardt Limited

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standing here and sharing that what are we going to do with Zaynich for the India and for the emerging market?

As you heard from Zahabiya that Zaynich is the first and the novel beta-lactamase enhancer, you know, and that is kind of comes with a very comprehensive coverage for your carbapenem-resistant gram-negative organisms. And most importantly, and I will emphasize, most importantly is that it kind of focuses of all resistant mechanisms, particularly which are relevant on India as well as in emerging markets.

With the two milestone approvals that we have, which is US FDA and the CDSCO India, I think with the composite cure superiority versus the standard of cure which is Meropenem, 20% superiority composite cure rate, and the kind of immense 200-plus publications that we have had, I believe, we believe that Zaynich is a once-in-generation, once-in-a-generation of patient impact, humanitarian, and commercial opportunity. And it is extreme my pleasure that I talk to you that what are we going to do with it in India.

So, let me begin with a, very often, I think many of you are investors and shareholders and there is always a question, what's the market potential? It doesn't come from a playbook of ma

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analysts. It comes from what's happening today in the ICUs of India. This comes from the surveillance networks that Government of India or ICMR runs and which talks about that there is tremendous burden of patients and unmet needs. If you see here that there are 600,000 to 700,000 patients which have gram-negative infections and there is a likelihood that there are 35% to 45% of them who have carbapenem resistance.

This is exactly where Zaynich works. And if you, and of course, I would have to caveat it that this comes from a surveillance data, so there would be variations in regions, there would be

variations in hospitals and tertiary care centers, but this is broadly what it, where is the burden today and the unmet need today that Zaynich will fulfill.

And without getting, and you don't have to read those tough organisms and pathogens on the right, but just to give you a figure that these organisms, which are the gram -negative organisms, are only growing becoming more resistant. They're becoming more rigid and more adamant to get treated.

And if you look at it here, Klebsiella is one the most prevalent one, whereas there is Acinetobacter which is the most resistant one, and then we have Pseudomonas which is very , very highly and alarmingly rising. So, the fact is, and unfortunately, that in India and in many parts of the world, the rising CR cases and the resistant trend is real. It's going to stay and that's what it is. So, that's what we are doing.

And this I really want, ladies and gentlemen, for your attention here, that it takes usually for an US FDA product, for an innovative R&D product, to take 2 to 10 years to reach a market like India, to reach any market like emerging markets. And we want, we as Wockhardt, we want to break the cycle.

We are challenging the conventional launch playbook of innovation because we are bringing it in US, we are bringing it parallel in India, we have submitted for European approvals, and we

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would be pursuing for emerging markets. And this is significantly a very distinct starting point for an innovation.

I'll now double -click and talk about that how will the playbook for us for Zaynich launch in India will look like. Very, very driven and guided by our Chairman's vision for access -first mindset, and that's what we do as for bringing Zaynich 's access to t he countries. So, first of all, we would drive accelerated market penetration.

We will go where the patients are. And that's what we'll need that we partner with hospitals, we partner with healthcare physicians to build the right experience so that the pa tients get the product. The other one, very important, is to eliminate the price as a barrier. Mostly you see that for innovation, price becomes a barrier, and driven by our vision that we have as Wockhardt, we will eliminate price as a barri er.

And at the same time, we would work on something what we believe is supremely important, is precision antibiotic stewardship. And precision antibiotic stewardship in simple words is to get the right patient, the right drug with the right diagnosis. An d this is where we will work with physicians in the country because Indian physicians deal with very complex infections and they really have difficult times.

And that's what will help us to make it the right standard of care. And last but not least, I alr eady mentioned that as a country, we have a very strong AMR policy, we have a strong research and surveillance network, and we will, as a very, very committed player in this, and both as an R&D and a commercial player, we will continue to participate in br inging management excellence, bringing patient -centric standards in the country.

A little bit more. Yeah. So, as I said that while we bring this model, please remember that we bring precision, the right kind of precision stewardship, also at the

same time, we start the kind

of keep continuing to build the experience and the real -world evidence. This is a little bit more about the India launch plan, and I would like your attention to wal k through it with me a little bit.

Zaynich works, has to g o to the tertiary care hospitals where you really have hospitals with large hospitals with multiple large number of beds of ICUs. We are focusing to go as per the carbapenem resistance burden. So our anchor is go where the patient needs you the most . So, that's where is the pyramid.

We'll go to the large hospitals and then we will kind of start expanding to the hospitals which is moderate and then we kind of go later. The one unique approach that we would have, that Zaynich , we will have access to b oth private hospitals, which is taking care of a large burden of ICU in the country, as well as the public access because that's what our, for us, access is real, it's not a theoretic theory on the paper.

Couple of things which we are building, and Zahabi ya is very nice to see that how strong a US team we have built with experts, and in India we are building, we have built a very strong expert team for infectious disease, especially people who have come with deep experience. And these are the couple of pillars which we are building.

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First of all, working with the experts in the country so that we could co-create the right usage guidelines because the physicians can then adopt the product and use the product more effectively for patients they need it. One unique thing which Wockhardt being an India R&D is that we are able to also create a 360-degree R&D-enabled scientific capability.

I have most revered my Dr. Sachin Bhagwat and Dr. Mahesh Patel here. I think they are able to create that R&D-enabled scientific ecosystem in the country. India-specific price point, as I said and maybe I'm repeating myself maybe 3x in 10 minutes, that that's very important that we unlock the access and remove the price as a barrier.

We are going to work, on public market, that how in India also that cost-effectiveness data helps to adoption in the public market faster. And then parallelly, as I said, that real-world evidence and patient access program is fundamental to how we do this. So, these are the six pillars. And who will do this is, let me give you the confidence that I'm very privileged to have a very, very strong team.

If I move to talk about a little bit more and double-click on emerging markets. Of course, India has a leading AMR burden, but emerging market contributes to 70% to 80% of that burden. There are significant amount of cases that you can see, I'll not repeat the numbers, but also the resistance, the rise in resistance is very, very concerning.

It's a large market size and aligned with our philosophy of breaking the conventional launch playbook, we would launch in seven to eight markets with high carbapenem resistance burden in the next 18 to 24 months. And these are markets in Latin America, Eurasia, GCC, South and Southeast Asian markets where we are playing.

India would serve as a great blueprint. We will learn, we will be able to build this capability. As I sum up, it's important that what we are doing as Wockhardt's global ambition for Zaynich, I think you can see that India and emerging market will be a strong compounding effect of what we are doing.

This kind of a cycle of a significant patient pool and a patient access gives us a very, very formidable and a very large size commercial opportunity with unparalleled patient impact. So, we believe that we are building a patient-centric business and operating model, delivering sizable, sustainable, and profitable revenue model for Zaynich.

And I'm very, very glad that we are here to talk more about this. Millions of patients in emerging markets are waiting for the right

antibiotic. We have Zaynich. Every day without access is a life at risk from carbapenem-resistant infections. So, it's so important that we kind of work at speed, work with agility, and as our Chairman always says, we are a learning and an agile organization and we continue to build with that in these markets.

And I'm extremely thankful for you coming here and listening to us. And on that note, as we are transforming as Wockhardt, I would like to share with you a small video on Wockhardt's transformation. Thank you all of you again.

[Video Presentation 0:54:12]

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Vikram Singh: Thank you, Ms. Annapurna. I'm Vikram Singh from Wockhardt, and I'll be moderating the Q&A segment. Before we start, I'd like to lay down some rules that we will start with the participants present at the venue first. If you would like to ask a question, please raise your hand and one of my colleagues will reach out to you with the microphone and you can ask your question. For participants joining us virtually, please continue to submit your questions in the chat box. We will take up as many questions as time permits. Also, may I request everyone to keep their questions brief so that we can accommodate a large number of participants. Yeah. Yeah, third row, gentleman in the blue.

Tarang Shah: Yeah, hi everyone, this is Tarang Shah. First of all, it's a very proud moment for all of us, the way Wockhardt has laid the roadmap. My question over here is, as we move from approval to commercialization, what do you feel will be the top two to three execution risks or challenges over the next 12 months, and how do you plan to mitigate them? Thank you.

Habil Khorakiwala: A very incisive question. I think one thing is very clear that we are in a process of creating entirely a new business model. Because when you have generic, biosimilar, it's a different kind of a business where commercial elements, other things are there. And the capabilities and competence required to communicate about new drugs like Zaynich is entirely different.

So, that is the challenge, biggest challenge we have in next 12 months to 18 months to build and create this organization. And as you would have seen in Zahabiya's presentation, the, and that is true outside India, but in India also, that what we have really focused is on maintaining the leadership and intellectual capital of science and medicine in-house.

And the operational part outside India and US and other parts of the world, we would have a

partner who would manage many of these things. So, it's in your financial terms you say investment, I mean, asset-light and capital-heavy. Yeah.

Tarang Shah: Okay. Thank you, sir.

Vikram Singh: Next question.

Siddharth Mehta: I'm Siddharth Mehta here. Just congratulations to Dr. Habil Khorakiwala personally. I'm an investor and I've been a shareholder of yours patiently for 12 years and been following the development and the R&D that you've put behind Zaynich. So, my personal congratulations to you, sir. It's a crowning achievement for your entire lifetime.

My question is, can you give us some sort of projections on what you're looking in terms of capex and as well as revenue to be had, while pursuing these products across America, across India? Can you give us some sort of idea about what the commercials might look like, what is your best guess at this point?

Habil Khorakiwala: If I understand your question, what you are saying, what will be in future, right?

Siddharth Mehta: What would be the revenue that you expect from different segments in the next six months, one year, two years?

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Habil Khorakiwala: There are three or four elements are there. What you have seen are revenue numbers from coming from a research part of the organization outside India. One would see a slow increase in

revenue because a new molecule doesn't happen suddenly. So, it takes doctors, institution time to make these understand the molecule, start using.

So, one would not see a very significant upside in revenue basically in next, it will be there as a slow increase. It's like a hockey stick. Wait for 12-18 months and then you would see a very different trajectory of business. That is one.

Second, in terms of capex, I think we have a normal capex would be there in next three years of about INR200 crores to INR300 crores. And we would be putting these into the capacity for biologicals primarily. And the other would be our R&D spends, which we have been incurring over the years, that would continue at the more or less same space as a percentage of sales. And you can expect a much faster growth than what you have seen in last 12 months or 18 months because these will add the revenue and the profit will improve accordingly.

Siddharth Mehta: How much capex do you see for pushing sales, for developing the markets, for business development, promotion, that kind of thing?

Habil Khorakiwala: There is no capex, there would be a revenue cost. So, what we expect in first 12 to 18 months, there will be slight negative impact on that because you have to establish the organization, there is upfront cost. But we expect to breakeven in 12 months or maximum of 18 months in terms of bottom line. And I think then one would see the hockey curve rising because the rest would be then a large improvement in bottom line and profitability.

Siddharth Mehta: Thank you.

Vikram Singh: Yes, gentleman on the second row please. Second row, please. Yeah.

Questioner: Sir, I believe it was not, but 18 months ago that we were at this very space. And let me just reassure you that the developments of the company are astronomical. So, again, congratulations to you, the scientific team.

Sir, I have three questions. The first question is under the Department of Pharmaceuticals' PLI scheme, Nafithromycin, I believe, has benefited from a 10% PLI. I'm wondering if Zaynich would benefit from the same. And if so, have we registered for that?

The second, if you could throw some light on the China strategy. And the third question is, I believe we have a certain incentive from the government of the United Kingdom where we have, I believe, GBP 20 million per annum. But I believe that, that is some sort of a blanket incentive, whereas they believe that we shouldn't be manufacturing excessively or something like that. So, just your thoughts on these three points, sir.

Habil Khorakiwala: Okay. The first question, government have recognized and some funding has come. But from our company's perspective and R&D spend, it is a very small percentage of it. So, it's welcome

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from the government, but it is not making any significant difference in what we would do otherwise.

The second question was China. China is a very complex market, and our clinical trial did have Chinese patients, but it was not large enough because if we had completed the Chinese recruitment, our whole approval would have been delayed by 12 to 15 months. So, we just closed

the -- and we are doing in parallel a small China study.

And we hope to file it in maybe in due course of time, it is not that important. And then probably we'll see how to approach, maybe we find a partner or a distributing existing company for the Chinese market basically. And your third question was?

Questioner: UK?

Habil Khorakiwala: UK. UK has done extreme. It's a very forward-looking country in terms of helping antibiotic research. And what they're saying is they will, they've developed a subscription model. And you have to get your molecule approved, subscription up to GBP 20 million. They will give you every year irrespective of, and we have to supply goods to them, whether you supply goods

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worth GBP 1 million or GBP 25 million, but they will give you a fixed GBP 20 million for three years.

So what it does, initially, as I mentioned, it's a slow increase, and that is why they actually support you to introduce new antibiotics because that is a crying need. Thank you for that question.

Questioner: I believe they call it the net ...

Habil Khorakiwala: I must tell you one thing, since you mentioned Mahesh Patel and Sachin's name, I must ask all of you to recognize them. It is their genius, innovativeness, and a great leadership in research which sustained the organization for these 25 years.

And I can add that top leadership in various discipline which we have, because it's not only one discipline, you need chemistry, microbiology, toxicology, so many other areas, pharmacokinetics. So, we have established this, that today our top leadership is same as it was 25 years ago.

Questioner: Hi, sir.

Habil Khorakiwala: Sorry.

Vikram Singh: Sir you can use the mic. Yeah.

Habil Khorakiwala: Mahesh would like to say something, comment on that. Yeah, yeah. It is your life purpose or anything you say.

Mahesh Patel: The life purpose -- can you hear me?

Habil Khorakiwala: You go ahead.

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Mahesh Patel: So, I think the purpose was only to find something with which everybody benefits. And the current problem of AMR, which has been, I mean, going on for last almost 15 years, was in a way creating a more and more enthusiasm in our team that we must do something where we can really make a global impact.

And I think that dream has come true through Zaynich. I mean, Zaynich, just to give you a metaphor, Alexander Fleming discovered penicillin in 1928. That drug was meant for gram-positive pathogen. Zaynich in a way represents another penicillin for gram-negative infection. So, it has happened after a gap of almost 80 years.

But challenging science does take time, and which you must recognize. Challenging science doesn't happen overnight. And this is a really completely new mechanism-based drug. And we ourselves took a 15 years to understand this drug. And believe me, still we are understanding many new aspects of this drug. So, it is going to be a lifelong journey for all our R&D team to understand Zaynich more and more. Thank you very much.

Habil Khorakiwala: Sachin, would you like to say that Chief Scientific Officer, not only Mahesh that he is now mentor, but he has created a successor will carry on the leadership.

Sachin Bhagwat: I mean, I would just add the point that our team is now basically battle-hardened. So, in last 25 years or 30 years, we have developed a, I mean, a very reasonable confident team who can now take on the challenge of AMR even in the future.

I mean, one of the important areas that we have been able to do in over years is develop a culture of science. And that takes the longest time to do. But once the culture is there, then I mean, you have a sustainable platform where the new drugs can come in.

So, from that perspective, we are in a good position. And with respect to some of the recent developments, I mean, one area where we wanted to really do well was regulatory interactions and some of the regulatory successes. So, we believe, I mean, that is also on the way and it is happening.

So, with that, we are in a much stronger position and we would be, I mean, for sure, living the dream of Chairman where we would like to maintain a leadership in antibiotic research for many more years to come. Thank you.

Vikram Singh: Yes, gentleman in the fourth row in white.

Questioner: Yeah. Thank you so much. Seeing the passion, perseverance, it's very heart-warming. Thank you also to your US team. I don't know what time it is over there. Happy to see them here. My

question is, I read in the media that Zaynich will be manufactured by Wockhardt in India, but I

also saw in the presentation Zaynich for the US will be made in Europe in a US FDA approved facility. Are both true, that for the US Zaynich will be made in Europe and maybe for emerging markets it will be made in India?

Habil Khorakiwala: Definitely . Europe as we already said , and we have filed our application from the manufactured in US . So that was basically a de-risk strategy. So, from that point of view, the Wockhardt Limited
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product is approved, the obviously site to our approvable site. FDA did a tremendous due diligence in this six months, even they spend.

According to them, they must have given their physical time visiting our facility about two to three months, plus continuous dialogue with our team. As far as an emerging market is concerned, India definitely we will manufacture from India.

Emerging market, we have an option either to manufacture from India or supply from Europe.

That will depend on the timeline we are reviewing, that if you file it from Europe manufacturing, we could save six to nine months. And that is the kind of issue we are looking. And for a new molecule , it is important the time is more important than cost of manufacturing.

So, we are debating, we have to make sure the availability is also there. But that will come much later, and we expect European approval by end of the year. We have submitted simultaneously about three, four months later. So, we expect they will take another four, six months before we get an approval, six. And then obviously, we will have both the files available for filing to all the emerging market.

Vikram Singh: Thank you, sir. Next question. Sir, on the second row, please, in blue shirt.

Questioner: Congratulations on your success of this new entity. I just wanted to understand, in 1995, you did the GDR and raised about \$75 million. Am I audible?

Habil Khorakiwala: No. I can't follow you.

Questioner: In 1995, the company raised a GDR of \$75 million.

Habil Khorakiwala: Okay .

Questioner: And there has been a long journey of 30 years , and probably your scientists and your research team must be working. Can you just walk us through what all worked and what all did not work?

Because till the time Wockhardt 2.0 came, which was a year back, actually we had all forgotten about this company. Thank you very much. Should I repeat the question?

Habil Khorakiwala: Are you talking about the company or research program?

Questioner: I'm talking about the company because as a shareholder we are only holding the shares of the company.

Habil Khorakiwala: Yeah, I think a lot of things worked in terms of we may acquire companies in UK which is very much there in Ireland. We acquired some company in France which was not a good acquisition.

So, that is where that was there. And I think we were, we established manufacturing for biologicals, insulin others during this period. And that is a and therefore the whole biological infrastructure we have created in terms of our research capability, in terms of manufacturing capability in API and also developing the pen you require for insulin, which is our patented pen.

So, device is a very important part of it. So, that is where that infrastructure is there. So, that is where we are now taking a very focused approach on biologicals with diabetes portfolio only.

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And some of the derivatives we did in late 80s, that is where we got in our financial challenge and trouble.

And we did well in some of our products in US like metoprolol, which gives us lot of liquidity in short term. And then we again had some challenge, we divested some of our businesses over a period of time. And ultimately, in all these areas one thing was very clear that our drug discovery program for antibiotics, which was going ahead, it slowed down a little bit in between, but I think that is where we

stayed our ground.

No normal person would continue a long -term research which are not going to give you immediate something in such kind of financial stress which went through which all of you know. And today we are in a fairly healthy position, our normal business is improving, we cut down on US generic.

I'll give you reason why we went out of US generic. We found it has become very competitive.

And our whole approach was to go into area where we have a very big differential. And we thought it is going to become – if we tried it out, we saw it is burdening, we are losing money, there was no sense getting into area.

And many times the company have a big challenge of growth and which we had over these years, that's why we went on acquiring. But we are in a very different position today as an organization. We do not have a challenge in terms of growth for next 10 to 20 years. We don't need to acquire anything, excepting we can revisit the research or some other part. I don't need to acquire anything for growth.

The growth is inherent in the organization because we have a series of molecules. The second element I must highlight, that if you look antibiotic portfolio, okay, definitely Zaynich US FDA is a great achievement, but what does it mean? We have six of our molecules US FDA has given a QIDP status.

There is no other company in the world who has more than one or two. So, it says that we are the most successful antibiotic drug discovery organization anywhere in the world. 25 years I thought that antibiotic in a competitive sense we can compete. You know about eight companies started, today we are the only one who have completed in the industry.

And as a result of this, I believe today that pharmaceutical, big pharmaceutical companies, because they don't have a research pipeline in antibiotics, so they would be not enough business in 5, 10 years from today. So, what one sees is a real clean white space worldwide in antibiotic.

We have a strong R&D organization, we have the program which we will continue to go ahead with that. And we are building up organization in next 2, 3 years to sell antibiotic worldwide.

So, we are walking into a space and we will work on this advantage as far as the our research program is concerned in antibiotic space.

Questioner: Second question if I may, this is to Dr. Patel. Do you think this is one of the jackpots which you have hit or there are others which are coming in the pipeline?

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Habil Khorakiwala: Sorry.

Questioner: I am asking Mr. Patel that this is one of the jackpot which you have hit or there are other which are coming in the pipeline?

Habil Khorakiwala: I can tell you one, I can tell you every product of ours is a jackpot.

Questioner: Thank you. Thank you very much.

Vikram Singh : Thank you, sir. Gentleman in the third, yeah.

Questioner: Thanks for the opportunity. Sir, my question you have already answered just now to exit the US generics market. So, what is the reason behind it? Is it because of the conflict with the patented products or any other colours you can give?

Murtaza Khorakiwala: So, when we took this decision about 3 to 4 years back, the US business was about 5% of our entire portfolio. It wasn't profitable, it was having losses. Because also we had the US FDA import alert in 2012 and 13. That was weighing on our mind and we did not want to invest in a business which is so insignificant to us, making a loss.

We had other opportunities where we thought we could invest and the returns were going to be much better. And at the back end, we were developing our entire antibiotic portfolio. So, what we rather did is we exited the US generic business which was insignificant and we focused on UK and Ireland, which is about 50% of our entire business in growing that business. And we double down on biotech where we are focused in the

diabetes space and with a relatively limited competition and that's where we are investing heavily in the future and that will have very good growth going forward.

Questioner: So, are you revealing to enter again because now you have the all this 483 form and US FDA you have got the approval ? So, again, they might be giving for other generic products ?

Murtaza Khorakiwala: Yeah. So, this, the generic business that was there, we are re-entering, in fact, we already have the team in the US with the innovative portfolio. And the innovative portfolio is de-risked in the sense that we are not manufacturing it ourselves. We are using suppliers and manufacturers in the Europe who already have all the approvals. So, from that point of view, the manufacturing GMP, quality, FDA, all that is completely de-risked. And for India, we are making it in India.

Questioner: Because so many companies like Sun Pharma and others, they have made fortune out of this branded US generic business . So, it's a very lucrative one?

Habil Khorakiwala: See, every organization has to choose what is the best way to forward. So, I think that is a choice we have made. And this is not the only choice possible. There are so many options available in the industry and you will find different business models, different organizations adopt and they are all successful.

Questioner: Thank you, sir.

Vikram Singh: Thank you. Gentleman in the third row in green.

Questioner: Okay, I think it is working now. Dr. Khorakiwala, Dr. Patel, it is absolutely surreal what you have done for us as shareholders, for the country. I came from Delhi, came today just to see you in person. So, it is a fabulous moment for me. I just wanted to check and find out, since you were able to secure a waiver for Zaynich Phase 2 trials. Is there a possibility that you'll be able to secure a waiver for Odrate Phase 2 trials? Because I understand both of them are going to be complementary and I also understand if they both are introduced in United States, they will possibly take over every hospital by storm. Thank you.

Habil Khorakiwala: A very good question. We got a waiver with FDA for Zaynich because cefepime dose we did not change what was approved by FDA for usage. As far as Aztreonam combination is there with Zidebactam. We have changed our dose from 1 to 2 grams. And I don't believe we would get a waiver. Even if we don't get waiver, from our point of view, we would want to do Phase 2 trial before we proceed.

Vikram Singh: Thank you. Let's take one question from participants joining us virtually.

Habil Khorakiwala: Yes. No virtual.

Vikram Singh: I am so sorry. Please sir.

Habil Khorakiwala: Put it on and give

Questioner: Thank you so much for the opportunity. Well this was a just wait, much shorter than what you guys have done for your drug. Many, many congratulations to Dr. Khorakiwala, Dr. Patel the entire team for a momentous achievement. From your presentation you mentioned that in the final approval, FDA approval. They have given a mention of drug for carbapenem resistant, for carbapenem resistant cases for HA BP, VABP. Does this mean that you don't need to do any trials for that particular indication and how does your strategy evolve with this level indication?

Mahesh Patel: You are asking about trial on a carbapenem resistant pathogen. See in the label what they have said I will explain you they have clearly mentioned that these particular drugs due to its new mechanism covers variety of resistant mechanism including various because there're carbapenem resistant mechanism also there are six resistant mechanism.

So it is mentioned that this drug covers all the resistant mechanism in a particular section of the label it clearly mentioned that and it is attributed, but in order to get an indication. So all gram negative resistant mechanism

are mentioned in our prescribing label this drug is active against that pathogen.

However, I mean we are going to do one more study which is a very important study called hospital acquired pneumonia and ventilator associated pneumonia where we are going to intentionally enrol the patient with carbapenem resistant pathogen and that will lead to a further expansion of indication as well as much more clearer coverage will be mentioned in that label.

Questioner: So essentially just a follow up to this essentially until we do that study we are not going to be pushing for off label usage for these cases?

Management: The label mention – the indication approval as per CUTI and the label mentions the pathogen which Zaynich has covers, which covers Enterobacterales and Pseudomonas. So it has given a mention of those pathogens for clinicians to interpret and make their judgement. For any other indication we still have to do a trial to get the indication approved which for HAP/ VAP , intra abdominal or anything else.

Questioner: One last thing if I may following on from the gentlemen before me. You have so Zaynich is the first drug that will enter US and you have Odrate to follow it maybe Foviscu as well at some point. So how does the strategy take care are you going to work on all the drugs simultaneously?

Habil Khorakiwala: To be very frank with you we have strategy only for Zaynich today and then we have to take a call which is next product we will go and go ahead and we don't believe any product can come less than 4 years.

Questioner: Thank you.

Vikram Singh: Thank you. Before we get back to the audience present here we will take one question from the participant joining us virtually sir. How much revenue is expected from Zaynich in India and US in next 2 years and what is the peak sales potential and when do we think the peak sales potential will be achieved in USA?

Habil Khorakiwala: I have mentioned repeatedly in last two, three days that globally we can expect a revenue overall about \$1.5 billion to \$2 billion. It is very difficult at this point in time to say country by country what will happen, but the fact is normally the US represents roughly 40% of our global revenue

for any area in our industry.

Vikram Singh: Thank you, sir. So, we have one more question. How will Emrok and Miq naf complement the growth of Wockhardt along with Zay nich? Also, are there any updates on Foviscu?

Habil Khorakiwala: Definitely these two molecules are already introduced in India and it will be a very important part of portfolio as far as India is concerned. And Foviscu, we are about to file it in another month or two, in another two, three months we are about to file to DCGI. And I think we should receive approval whenever DCGI gives approval, that is not predictable nowadays.

Vikram Singh: Thank you, sir. We'll now get back to the audience here in the atrium. I think ma'am on the second row.

Questioner: Congratulations to the entire team. Feels like a national win. Sir, I want to understand what is the expected pricing for Zay nich across India, USA, and Europe that we're expecting? And secondly, my question is how many patients are we targeting in say FY28, FY 29?

Habil Khorakiwala: See, the price point as far as the US is concerned of newer antibiotic for a daily cost is somewhere between \$1,200 and \$1,500 per day. And normally a treatment is about eight to 10 days, it's about \$10,000, \$12,000, \$15,000 a day. Our India prices will be heavily discounted to US because our approach is make it affordable or equally affordable everywhere. So, roughly we will have 75%, 80% discount to US pricing.

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Vikram Singh: Yes. Sir in the second. Third last row.

Habil Khorakiwala: You had a second part of the question.

Vikram Singh: Just allow us a moment

ent we will get back to you.

Questioner: And secondly, sir, the number of patients that we're targeting in FY28, FY 29 across.

Habil Khorakiwala: I cannot tell you short term what will happen. Long term we will get significant market share, about 20%, 25% of resistant cases everywhere. In India, we might get little bit better. Let us see.

Questioner: And sir, just sorry, one last question. Sir, you mentioned that you've partnered in USA for handling the marketing and sales distribution. So, what kind of cost do we expect per annum for our marketing and sales, if you can give an absolute number or as a percentage of our revenue?

Habil Khorakiwala: See, there is certain minimum cost when you start a business and organization, and that is what we have to incur at initial stage. Once you establish that organization, then the recurring cost is more or less same with normal 5%, 10% increase which takes place. And that's why when we are looking at all these aspects, I did mention in the first 12-18 months we might have from profitability point of view a minor loss, neutral, and that is space we would be. Afterwards, it could be a different scenario.

Zahabiya Khorakiwala: I'll just add to that. So, the cost is not hugely different whether we do it ourselves or an outsourcing commercial partner does it. The difference really is the operational bandwidth that we free up to be able to do more strategic and execution-related things. We don't want to too much get into the nitty-gritty of an operating model which is quite complex in the US.

Questioner: Thank you so much.

Murtaza Khorakiwala: Just a clarification, when Chairman said there will be a slight loss, he's referring to the Zay nich part of it and not the whole company. Because I saw a worried look on your face.

Vikram Singh: Yes, sir.

Questioner: Congratulations to Wockhardt team and especially scientific fraternity, and we are proud to have scientific fraternity and Wockhardt as a company in India. My first question is, could you throw some light on the patents for Emrok, Zay nich, and Miq naf? What's the patent validity, and is there any option to extend the patent?

Habil Khorakiwala: As far as Zay nich is concerned, the patent is, current patent is at '38. So, you have another more than 10 years to go for that. And there is also we are filing another few more patents, so we are obviously looking at extending the life of Zay nich patent beyond '38. And as far as Emrok and Miq naf is concerned, the Miq naf patent is there up to early '33 in India.

But any product we want to take in the western market, US and Europe, and because of QIDP status we got in US, we have already, for example, received a letter from FDA after approval that they've granted as per QIDP status extra five years. So, normally if a product, any product

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we introduce irrespective of patent expiry, minimum 10 years one gets either you go to US or any western market.

Questioner: So, 2038 plus five years?

Habil Khorakiwala: No, no. Not after patent. Total maximum exclusivity with the laws is 10 years. Now, if your

patent is longer, the patent will work. But if patent expires early, this minimum 10 years will be operative.

Questioner: So, there is any option to extend by proving the product that it can be used for many other disease?

Habil Khorakiwala: See, let us recognize these are new molecule and it is not easy to extend a patent unless something innovation take place. This ever greening which we talk about, which is in generic space with process and other thing, don't work in new drug discovery space.

Questioner: For Miquan and Emro, which is specifically targeted for emerging markets, so in I think.

Habil Khorakiwala: No, I think, let's stay focused. Our strategy fundamentally now onwards to whatever we have in Emro, fine, we are only entering with Zayn

in worldwide and no other product.

Questioner: No, it's not about worldwide. What my question is, so Miquan and

Habil Khorakiwala: No, we are not entering other than India, Miquan for example.

Questioner: Okay, that was my question. Only India.

Habil Khorakiwala: That's it. Only one product we are, because I think Zayn is our strong anchor. We want to go to every country and establish ourselves in a strong manner, and then we can see what products we would like to introduce thereafter.

Questioner: Are there any possibility for partnership deals in smaller countries where Wockhardt can not go?

Habil Khorakiwala: No. See, our strategy is very clear because partnership option we did consider over last 12 months. And then after various stages of consideration, we have come to a strategic decision to build the business ourselves because we are not a one-product wonder. We have a range of products coming out over the years. So, I establish an organization and we continue. Second biggest advantage people don't recognize, that if you're committed 100% to a cause, which we are, we'll do a better job in penetrating the market than a partner who is distracted with half a dozen products.

Questioner: My last two questions.

Habil Khorakiwala: Sorry, we can give other people chance.

Vikram Singh: Sir we will get back to you.

Habil Khorakiwala: Yeah, so we'll get back to you after.

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Questioner: A quick question. One of the pillars of the strategy for Zayn in the US you mentioned was HEOR, which is reducing the burden in the hospital and lower admission. Are we referring to Odrate? So, how would you distinguish between these two and what would be the opportunity sizes for Odrate?

Zahabiya Khorakiwala: So, I'll just explain that. HEOR is basically the hospital economic benefit that hospitals look at in terms of using one drug versus another drug. Because of Zayn's complete pathogen as well as resistant mechanism coverage, and because we know that a lot of infections are poly microbial with multi-resistant mechanisms, what we expect is that the re-admission rate will be lower. Today, the re-admission rate for cUTI is 20% to 25%. Every time a patient is re-admitted for the same indication, the hospital gets penalized financially. So, we expect that re-admission rate by using Zayn will be much lower because of the profile of the drug, and therefore the economic benefit to the hospital and the healthcare system will be better.

And also the length of stay. So, if the average length of stay of a patient is lower, then it is better for the hospital because the other costs are reduced. So, that's the second economic benefit. So, on both of these, we're going to be collecting data, doing studies on this, and really presenting the value proposition in addition to the clinical value, and we believe that we will be able to showcase that.

Questioner: We believe that, Dr. Khorakiwala has said that Odrate is around four years away. But best part for the Odrate as per our belief is that since it's a once-a-day product and no hospitalization is required, it is, so can Odrate become as high as Zayn?

Habil Khorakiwala: You are right. Absolutely. It is a great opportunity which is, because what happens once the Zayn we establish the presence in the institution, today anti-infective product use outside the hospital in United is about one third, 35% to 40%. And once a day is a great advantage, that's why this product is there. Clinically, actually from a pure science point of view, Zayn is a better product. But as a follow-on treatment or outside the hospital, once a day makes a huge difference in the western market.

Questioner: So, sir, to be more precise, is this a great opportunity as compared to Zayn after four years maybe?

Habil Khorakiwala: I don't think I'll be

able to comment on it because this is only US position. Second, the molecule in terms of benefit, there is no other molecule anywhere near Zayn. So, would it be better

than Z aynich even commercially, I do not know. But we believe that Z aynich would be a very successful drug.

Questioner : Thank you, sir.

Vikram Singh: Thank you, sir. Gentleman in the fourth row on the left -hand side.

Sanjay: Hello. This is Sanjay. Seems to be a very humble family background, this whatever the changes brings in India in this segment. I think that it is a great, great community support also you are

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doing on this area. I want to know, as the pharma sector or hospital sector are moving into insurance segmen t also, is there any plan of your side to entering into the insurance segment?

Habil Khorakiwala: See, our approach as a company is a very focused approach. Within our industry itself, we have taken certain calls, and they are very narrow calls, they are n ot broad calls. So, go outside the industry and do health insurance and thing is nowhere in our possibility, I can tell you.

Sanjay: Okay thank you. Most of the, most of the health chain and all these things, they are also.

Habil Khorakiwala: So, what ever ybody does, we don't do. That's our basic principle.

Sanjay: Thank you, sir.

Vikram Singh: Sir, in the on the right -hand side, fourth last row.

Dixit: Hi, this is Dixit here. So, you did mention about the peak potential of Z aynich in last couple of days ab out \$1.5 billion to \$2 billion. If you can throw some light in terms of Miq naf and Emrok over let's say three to five years, what kind of potential those two drugs have? And also if you can guide for let's say next three to five years, what kind of growth we expect from biosimilar business?

Habil Khorakiwala: Biosimilar, I think Dr. Murtaza will answer . First 2 products I can tell you we are not going beyond India. So, there is no comparison with Z aynich potential and whatever we are able to do in India, but we will do very well in India.

Murtaza Khorakiwala: I'll take the easier . So, as far as biosimilar is concerned, we had about 35% growth in the last year, and I think we will double our business within the next 24 to 36 months. We are also investing in increasing our capacity, and by doing that, we will double our capacity in the next 12 to 15 months. After two years, this is for the current products that we have, after two years, we will launch the pipeline pro ducts which are in development.

So, we have five products in pipeline. As I said earlier, this is about a \$6 billion or \$7 billion India emerging market opportunity, so we'll look at getting a significant portion of that. So, I think we are on a good growth path , and one would expect that to continue in the years to come, if not accelerate.

Vikram Singh: Thank you, sir.

Habil Khorakiwala: Someone was asking question her e and I said we'll come back. Yes , you there, yeah.

Questioner: So, this was regarding the comm ercial part for Z aynich , especially in the US and the European markets. What's your go -to-market strategy? You said you want to do it yourself, or are you going to partner with any kind of distributors over there who have their presence over there? What's going to be your roadmap for that?

Zahabiya Khorakiwala: So, for the US, we are doing it ourselves clearly. The, as I mentioned to you that we will segment the market, there are about 6,000 hospitals in the United States. We're going to segment the market in terms of historically where are the prevalence of the pathogens, which institutes are

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the early adopters of novel antibiotics, and which are the clinicians th at have these kind of patients.

That's how we're going to identify our early customer penetrati on strategy. But broadly, the idea is that

we want fast hospital penetration, means get it on the formulary quickly, and we want deep clinical adoption, which means we want clinicians, there are institutes where there is a larger prevalence of certain path ogens where anti -novel antibiotics are used mo re aggressively to save a life.

So, we will identify those institutions and those clinicians where they do have an existing approach of using these as a first, first one, two, three -year strategy. And then, of course, the awareness for Z aynich will build also to the entire institution. For Europe.

Questioner: So, in terms of institutions you mentioned, because this not being your forte, you know, you guys are into deep -dive new drug discovery, I think this is the first instance where Wockhardt is getting into this space in terms of, you know, identifying new customers and distributing as well. Have you identified any institutions in terms of areas or regions where they are going to be your

target sales or customers?

Zahabiya Khorakiwala: So, actually, the team that we have got on board, the US leadership team, the Chief Commercial Officer was part of cefiderocol, Fetroja, for about 10 years. Leo, who has come on as the head for market access, he has been in the anti-infective space for about 20, 25 years. Similarly, Dr. Dennis Deruelle, he's a practicing ID physician, very well, has worked in large healthcare systems in the United States.

And Sandy as well has similar anti-infective experience. So, the team we've hired understands the market in the antibiotic anti-infective space, that is why we've brought them on board. I just kind of gave you a snapshot of how we will go about it, but it is using all the expertise and the leadership that we have that and this is the approach that we will take to entering the market.

Questioner: One last question. Any percentage allocation you have or any thought process in terms of the target sales for the US and the Europe markets? Just a broad-based number.

Zahabiya Khorakiwala: So, I can tell you that historically anti-infectives, new anti-infectives that have been launched in the first three-odd years do about maybe 30 to 50 million collectively over that period of time.

And...

Habil Khorakiwala: Collectively, all the anti -- new anti-infective which is there in the US market, and if you take those four, three, four, five drugs, is about a billion dollar today. So, we are sitting on new antibiotic in anti-infective of a billion dollar all put together. So, obviously, we are not creating a new demand, we are meeting only -- need much better, much faster, better with a superior drug. And in price, we are as a strategy we are going to be neutral. So, we are not going to take a price premium. So, it's easy to switch over to a better antibiotic. So, that's a fundamental strategy.

Questioner: All right. Thank you.

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Vikram Singh: Thank you, sir. Gentleman in the second row, please.

Questioner: Just a quick follow-up. You had mentioned the word "we'd be getting a commercial partner" twice. So, I'm a little confused whether you're doing your own or you're getting a commercial partner?

Zahabiya Khorakiwala: So, I'll just explain. It's actually an operating partner. What it is, is that we're doing it ourselves. The commercial partner essentially is somebody who will help us with the logistics of hiring a sales force, whilst we will select the people, they will help us with those logistics. They will help us, they have the distribution in place, the 3PL distribution, so the distribution of the drugs happens through them.

A lot of the pharmacovigilance, that piece will happen through them. So, there are many, many aspects to a US launch which are different from what you do in India. And therefore, those nuances of the market which rather than us building our understanding on it, our capabilities on

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it, the idea is to get, and this is a common practice.

This is not that Wockhardt is doing this for the first time. All anti-infectives, this has been a practice in the industry for about 15 years now. So, it is very common to kind of start with a commercial partner, so that all your operating stuff is taken care of whilst the business is focused on in the early years. And eventually then you decide how you want to...

Questioner: Thank you ma'am.

Questioner: Good evening, sir. What is the roadmap and strategy for Wockhardt hospitals? It's been doing continuous losses since past many years. It's turned profitable this year.

Dr. Habil Khorakiwala: Hospitals, I think it's beyond the scope of this meeting.

Vikram Singh: Yes, gentlemen, the fourth, third row.

Questioner: Hi, congratulations. I just wanted to check, you mentioned cefiderocol. So, cefiderocol got the US FDA in 2019 and expires in 2033. And at this stage, I think if I'm not mistaken, it's probably doing like \$250 million in revenue. So, what makes us confident, that we might have a peak sale which is much higher than that?

Dr. Habil Khorakiwala: No, no. See, it's a business question. So, let's leave it. I'll answer it. See, you have to look at the molecule, what it is there. Now, cefiderocol has a lot of warning signals, if you see the label today, because it has a side effect. Second, does not have the same action as zidebactam, I mean, Zaynich. And also, the side effect profile is different, efficacy profile is different. So Zaynich is far more effective as a drug, even though cefiderocol cover all the organisms like Zaynich. But the delta difference is the effectiveness of the drug and safety of the drug.

Questioner: So would you say that those who are using cefiderocol ...

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Dr. Habil Khorakiwala : I don't think , let's not discuss a single molecule. If you have something, I think offline we can discuss.

Questioner: Thank you, Sir

Vikram Singh : Sir, in the fourth row.

Questioner: I think you mentioned some 89 compassionate use cases for Zaynich, right? Is that typical or is it stand out? The reason I ask is, might it be possible the drug will sell itself, mean, it? But it won't be that difficult to sell it if there's already so much compassionate use, hospitals are aware of it. Okay, can you hear me now?

Dr. Habil Khorakiwala : We can hear you.

Questioner: What I'm asking is, is it typical for a drug to have so many compassionate use cases before launch? Or is it exceptional? Is it rare?

Dr. Habil Khorakiwala : Actually, as an anti-infective is concerned, I don't remember any molecule at this level of compassionate use. We must recognize some of Indian doctors. From pure literature, they had approached for Zaynich. Medanta, Dr. Dubey . Dr. Dubey, he was the first one to write. And when a success, he saw , where a patient after three or four months got cured in a week or 10 days and went home. And obviously in a professional circle, then I think we went on receiving , and we still go on receiving every day so much.

So, from all these points of view, one thing which is very clear in India especially, where there's so much compassionate use is done. The knowledge of the molecule efficacy is with the entire community of ID specialists today. There are only 300 ID specialists. So, from that point, whatever pre-launch activity people do to get off the ground, which we will have to do in US, but in India it is done. So, we expect Annapurna and the team to start hitting sixes from the day one.

Vikram Singh : Thank you sir. We are almost reaching the end of our allocated time. Sir, in the second last row, I think you had some question in blue.

Questioner: Thank for the opportunity. My question was what's the internal ROCE you are targeting for the next two to three years ROCE for the Wockhardt, and are there any plans

to develop new

molecules in chronic management? This is acute management for chronic disease. These are the two questions.

Dr. Habil Khorakiwala : I can guarantee you one thing. We will stick to only antibiotic research for next 10 years.

Questioner: And what's the ROCE? The management is targeting?

Dr. Habil Khorakiwala : What is the question?

Questioner: What's the internal ROCE?

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Dr. Habil Khorakiwala : Please understand this way, if as a company I have invested \$800 million over the last 25 years, are you looking for ROCE on \$800 million or you are looking on a future investment? So, consider this is already investment met and the new drug has been discovered. So, on ROCE or \$800 million, I don't know what will happen, but in future you can yourself know.

Questioner: In one of your interviews, you said there is a \$1.5 to \$2 billion of peak revenue. So, there is a gray area. So, \$1.5 to \$2 billion per year are the cumulative of Zaynich ?

Dr. Habil Khorakiwala : Peak sales per year.

Questioner: Peak sales per year. Okay. Thank you sir.

Vikram Singh : As we have reached the end of our allotted time, we will now conclude the Q&A session. Thank you for all your questions and the engaging discussion. Over to you, Dilini .

Dilini Quadros : Thank you, Vikram. For our guests present here, refreshments are being served in the lounge area to your left. And we would be delighted if you could all join us. And on behalf of Wockhardt Limited, I would like to thank all of you, both those who are present here today and those who have joined us online. Thank you very much for your participation and continued interest in the company. We wish you all a pleasant evening. Thank you.